



THE SAFETY TORAH

*Sacred Rules for Sessions,
Getting Paid, and Getting Home Safe*

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The stories in this book are based on real experiences. Names and identifying details have been changed to protect privacy. Any resemblance to actual persons, living or dead, is drawn from the author's life and community.

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This text is dedicated to every worker who has ever measured their own dose, counted their own money, enforced their own boundaries, and made their own rules.

To the ones we lost — you are not forgotten.

To the ones still working — you are not alone.

We keep each other safe. We keep each other alive. We are the torah.

* * *

THE SEX WORKER'S TORAH ON CHEMSEX

Sacred Rules for Getting High, Getting Paid, and
Getting Home Safe

* * *

FOREWORD: WHY A "TORAH"?

A torah is a teaching. Not the kind that comes from above, handed down by people who've never lived your life. The kind that comes from *inside* — forged in hotel rooms, earned in apartments where the bathroom door doesn't lock, carved out of nights that went sideways and mornings we didn't see coming.

I have worked in Buenos Aires, Barcelona, Amsterdam, Oslo, Prague, Berlin, London, São Paulo, Miami, Bangkok, Brussels, Zagreb, Tel Aviv, Singapore, Copenhagen and places whose names I legally cannot mention. I have been the sober one watching someone else's night unravel, and I have been the mess on the floor while someone else held my hair back. I have counted money with pupils, the size of dinner plates. I have lost money. I have lost phones. I once lost an entire suitcase in a city I had arrived in four hours earlier — don't ask, but yes, it involved G, a rooftop, and a very confused Uber driver.

This is not a book about "getting clean." This is not a cautionary tale. This is not your therapist's pamphlet or your mother's worst fear printed and bound.

This is a survival manual written by someone who has been there — high, working, counting money with one hand and measuring doses with the other — and lived to write the rules down.

I've met girls who could run a five-hour chemsex booking like a Michelin-starred restaurant — timing, pacing, presentation, upselling, flawless exit. I've met boys who couldn't find their own wallet after one line. The difference between them? Rules. Written or unwritten, conscious or instinctive, the ones who survive and thrive in this economy have rules.

Some of these rules I learned the easy way: watching someone else fuck up. A girl in Barcelona — let's call her Lola — who accepted "party favors" as payment and ended up giving a twelve-hour performance for what amounted to forty euros' worth of questionable MDMA. I watched her do it. I made a note: *never be Lola*.

Most of the rules I learned the hard way: my own body, my own money, my own time that I will never get back.

There was the night in Berlin when I let a client pour my drinks because I was "too tired to measure." Woke up twenty hours later with no

memory, no money, and a bruised hip from what I can only assume was a disagreement with gravity. The client had "helped himself" to a discount — except the discount was my entire fee, and the help was to my unconscious body. That was the night Rule 11 became non-negotiable.

There was the afternoon in São Paulo when I got higher than the client because I wanted to "have fun too." Two hours later, he was consoling *me* while I cried about a cat I hadn't seen in six years, and I forgot to charge him for the extra hour. He was nice about it — most clients are when you're having a drug-induced emotional breakdown in their hotel room — but nice doesn't pay rent. Rule 4: born from that disaster.

The goal here is not abstinence. The goal is agency. Pleasure without prison. Getting high without giving away your power. Making money without making yourself disposable.

If you're going to do this work — and many of us are and will continue to — then do it like a professional. Do it with your eyes open. Do it in a way that lets you wake up tomorrow with your money, your dignity, and your next move still in your hands.

This is the torah. These are the rules. They might save your life. They might just save your weekend. Either way, they're yours now.

INTRODUCTION: THE CHEMSEX ECONOMY

Let's talk about money, honey.

I arrived in Barcelona in the Autumm of 2013 with two suitcases, no Spanish (Argentinian Spanish doesn't count — they look at you like you've invented a new language), and a WhatsApp contact who promised me the city was "full of generous guys." Reader, the WhatsApp contact was half right. The city was full of guys. The generous part was... negotiable.

Here's what I learned in my first month: the chemsex economy is not the same as the sex work economy. They coexist, they overlap, but they operate on different physics.

In the regular economy, you're trading time and body for money. Clean. Simple. The client pays, you perform, everyone goes home. In the chemsex economy, there's a third party at the table: the substance. And the substance has its own agenda.

Clients don't just pay for your body in chemsex bookings. They pay for *experience*. They pay for complicity. They pay for the fantasy that you're not working — that you're just two people who happen to be getting high and having sex, and oh, by the way, here's an envelope. The money becomes almost incidental in the performance, which is exactly how they want it to feel, and exactly why you need to be the one who never forgets it isn't.

I remember my first chemsex booking in Barcelona. A British guy in a penthouse overlooking the port. He offered me a line before we'd even discussed the rate. "Relax," he said, "let's just see where the night goes."

Red flag number one. I didn't know then. I know now.

"Where the night goes" is code for "I'm hoping you'll get high enough to forget you're working." The drugs are not a bonus. They are not hospitality. They are a *tool of negotiation*, and the client who offers them before discussing money is trying to move the negotiation to a field where you're impaired and he isn't.

I did the line — I was new, I was nervous, I wanted to seem cool. But I also did something right, purely by instinct: I stopped after one. I said, "Let's sort out the details first, and then we can really enjoy ourselves."

He blinked. He hadn't expected that. The line was supposed to dissolve my business brain. Instead, my business brain had done the line and was still asking for a credit card.

We agreed on two hours. It ended up being four — but by then, I had already understood something crucial about this work: the money conversation has to happen before the first molecule enters your bloodstream. After that, you're negotiating from a weaker position, and the client knows it.

This book is about how to navigate this economy without becoming its casualty. The money is better in chemsex work — sometimes double, sometimes triple the sober rate — because the risk is higher. The client isn't just paying for your time. They're paying for your sobriety, your sharpness, your ability to read a room and enforce a boundary. They're paying you to leave your most protected self at the door and enter their world.

That has a price. That price needs to include not just the hours you're there, but the hours of recovery you'll lose, the physical toll, the emotional hangover, and the premium for what you're giving up.

I've done the math across twenty-five countries and more bookings than I can count. A chemsex booking should pay at minimum 50% more than your sober rate. If you charge 200 for a sober hour, chemsex starts at 300. This isn't greed. This is accounting for the half-day of recovery you'll need, the compromised negotiation position, the legal risk, and the fact that you're providing a specialist service that most escorts won't touch.

And that brings us to the rules. The twenty commandments that separate the professionals from the cautionary tales. Let's begin.

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PART I: THE MONEY RULES

Money is why we're here. If you forget that, everything else falls apart. The drugs, the sex, the client's fantasies, the music, the "connection," the sunrise conversations that feel profound but dissolve by Tuesday — none of it matters if you walk out with less than you came in with, or if what you made evaporates before the high wears off.

I have a friend — let's call her Valentina — who works in São Paulo. Valentina is the most financially disciplined person I have ever met. She has a spreadsheet. She has tax returns. She once negotiated with a client to pay for her MBA, one

booking at a time, over eighteen months. I am not making this up. The woman is a legend.

Valentina taught me the first principle of chemsex money: *the client pays for the fantasy that you're not working, which means you have to work twice as hard to make sure you actually get paid.*

She also told me something I've never forgotten: "The drugs want your money as much as you do. They're a second client in the room, and they don't negotiate. They just take."

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RULE 1: Never Work for a Fixed Price

Let me tell you about Martin, a German escort I met in Berlin who was, frankly, a disaster and also my favorite person to learn from.

Martin charged a flat rate: 500 euros for "the night." The problem with "the night" is that it has no definition. Is it six hours? Eight? Twelve? Until the drugs run out? Until someone passes out? Until Tuesday?

I watched Martin accept a booking at 10 PM on a Friday. The client had G, Tina, and an apparently unlimited supply of both. By the time I checked my phone Saturday evening, Martin was still in that apartment, still high, still "finishing up," and still had only 500 euros in his pocket. That's 500 euros for nearly twenty-four hours of work.

Congratulations, Martin — you just worked for less

than minimum wage while providing premium fantasy services.

"Martin," I said to him later, over the saddest brunch I've ever witnessed, "you know you could have made three times that, right?"

"But he was so nice," Martin said, staring into his orange juice like it contained the answers to the universe. "And we really connected."

Martin. Baby. The connection was the G. The G wants you to connect. The G wants you to stay forever and merge your consciousness with the furniture. The G does not pay your rent.

Fixed price is an invitation to be taken advantage of — not always maliciously, but systematically. The client who pays a flat rate will stretch time like it's elastic: "just five more minutes," "one more line," "let me just finish," "oh my God the sunrise is so beautiful, let's watch it together." Before you know it, three hours have become eight, and your effective hourly rate has dropped from "professional" to "intern."

Charge by the hour. Always. No exceptions. Even if you "really like" the client. Even if "it doesn't feel like work." Even if the drugs are telling you that you've found your soulmate and money doesn't matter between soulmates. Money always matters. Your soulmate can pay your hourly rate like everyone else.

Here's the thing about hourly rates that nobody tells you: they actually make the experience better for both parties. When the client knows the clock is running, they're more present. They don't waste time. They're clear about what they want. And you — you're not secretly resenting them for every minute past the unspoken deadline. You're relaxed, because every minute is a minute you're being compensated for.

Fixed price = resentment + exploitation. Hourly = clarity + mutual respect.

This is not about being greedy. This is about respecting your time as the finite, irreplaceable resource it is. You can always get more money. You can never get more time.

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RULE 2: Only Get High for Dates of 3+ Hours

Here's the math they don't teach you in school, and by "school" I mean "the WhatsApp group where we all complain about clients."

You do a two-hour booking on Tina. The date ends. You're still high — for potentially 48 more hours. Now what? You're buzzing, you're horny, your impulse control is on vacation. You order more drugs for the comedown. You order food delivery because cooking is for sober people. You take Ubers everywhere because your legs feel like they belong to someone else. You call up a friend because you're

still going and it would be a waste to come down alone. You book a hotel because you don't want to go home yet. You Venmo someone for "party supplies."

By the time you finally crash, two days later, you've spent everything you made on that two-hour booking and then some. You've effectively *paid* to work.

I learned this lesson through a girl named Pilar, who worked the London circuit and had the metabolism of a hummingbird. Pilar could do a two-hour booking, make 400 pounds, and spend 600 pounds in the following 48 hours on "maintenance."

Maintenance, in Pilar's vocabulary, included: more drugs, room service, a new outfit because her old one "felt wrong," a taxi to Brighton because she "needed to see the ocean," and an impulse purchase of a designer bag at 4 AM from a 24-hour shopping website I still maintain should be illegal.

"Do you still have any of the money from Tuesday?" I asked her once, on a Thursday.

"I have the bag," she said. It was a very nice bag. It was also not rent money.

The 3+ hour minimum solves this problem mathematically. A four-hour booking at your chemsex rate should cover: the substance itself (if you're providing), the time you're there, the recovery time you'll lose, a buffer for post-booking

expenses, AND leave you with actual profit. If the math doesn't work, the booking doesn't work.

Think of it like this: every hour of work you do high requires roughly two hours of recovery. A two-hour booking = four hours of recovery = six hours of your life. A four-hour booking at a higher rate? Same recovery time, double the money. The economics are not complicated once you actually look at them.

Shorter date? Stay sober. Save the party for bookings that justify it. Your future self, the one who's crashing on a Tuesday and looking at her bank account, will thank you.

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RULE 3: Drugs Are Never Payment

I want you to imagine a scene. You're in a beautiful apartment in London. The client is charming, well-dressed, and has what appears to be an unlimited supply of very good cocaine. "Don't worry about money," he says, waving at the table. "This is all for us. Just enjoy."

Three months later, you're telling this story to a friend and she asks the question you've been avoiding: "So... he paid you, right?"

Silence.

"Right?"

The thing about drugs-as-payment is that it's never a fair exchange. The client is offering you something

whose value is subjective, variable, and entirely determined by *his* scale. That "incredible MDMA" that was supposed to compensate for your fee? He probably paid forty euros for it. You just worked three hours for forty euros.

Drugs are not currency. They are not compensation. They are, at best, a bonus you offer to your most exclusive clients — the ones who already pay premium rates and who understand that you breaking your sobriety for them is a *privilege*, not a given.

I had a client in Barcelona who actually understood this. He would book four hours at my full rate, show up with his own supply, and say, "This is for me, but if you'd like to join, you're welcome." Then he would pay me in cash, at the beginning, before any substances were introduced. I didn't even have to ask — that's just how he operated.

I asked him once why he did it that way. "Because I want you to relax," he said. "If you're worried about whether I'm going to pay you, you're not present. And I'm paying for you to be present."

THAT is a client who gets it.

The ones who don't? The ones who offer "party favors" as a substitute for cash? They're not clients. They're drug buddies who want you to perform emotional and physical labor while they pretend the transaction isn't happening. They want the girlfriend

experience with none of the boyfriend responsibilities, chief among which is *paying you*. Make it clear from the beginning: "I don't accept substances as payment. If you'd like to share what you have *after* we've settled the fee, that's a conversation we can have." The ones who balk at this are doing you a favor — they're identifying themselves as time-wasters before you've invested any time.

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RULE 4: Never Get More High Than the Client

Let's talk about The Night I Cried About a Cat.

I was in Oslo. I had been working for about six months at this point, long enough to think I knew what I was doing, not long enough to actually know what I was doing. The client was a regular, someone I trusted. He had excellent coke. I had excellent confidence. We were both, in retrospect, about to make excellent mistakes.

The booking was going well. The client was in a good mood. I was in a great mood. The coke was very, very good coke. And somewhere around hour three, I crossed a line I didn't notice until I was already on the other side.

I was higher than the client. Significantly higher. Catastrophically higher.

And then I started talking about Mishi, my childhood cat. Mishi had been dead for six years. I had not thought about Mishi in at least five. But apparently the cocaine had unlocked a vault of feline grief that I had been unknowingly carrying, and now this client — this paying client, this man who had booked a fantasy experience — was getting a front-row seat to a thirty-minute monologue about a dead cat, complete with tears, photos on my phone that he did not ask to see, and a detailed analysis of Mishi's final days.

The client was nice about it. He held me. He said supportive things. He also got two extra hours of my time without paying for them, because I was too busy processing cat trauma to look at a clock.

When I finally came down, I realized three things:

1. I had spent the last two hours providing free therapy instead of paid companionship
2. The client now knew more about my childhood than my actual therapist
3. I had forgotten to enforce every single boundary I had set for myself that night

You are there to make money and show the client a good time. The drugs are tools — they're only as useful as they help you reach those goals. The moment you're higher than the client, the power dynamic flips. You become the entertainment, and not the kind you're being paid to provide.

When you're the one who's too gone, you can't: track time accurately, guard your boundaries, remember your rates, keep your belongings safe, read the room for danger, notice when the client is getting uncomfortable, or execute a graceful exit.

Stay one step behind the client's high. You're the captain of this ship. Even on a cruise, the captain doesn't get drunker than the passengers. The captain stays alert enough to steer, even while participating in the party.

This doesn't mean don't enjoy yourself. This means *control* your enjoyment. Sip, don't chug. Measure, don't guess. You can always take more — you can never take less. And if you feel yourself slipping past the client's level, slow down. Take a break. Drink water. Excuse yourself to the bathroom and splash cold water on your face. Anything that buys you ten minutes of recalibration.

Your job is to be the person who's having fun but still in charge. The client is paying for you to be that person. Be that person.

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RULE 5: You Are Not a Dealer

I once knew a boy in Berlin — let's call him Felix — who thought he had discovered an infinite money glitch. He would buy drugs in bulk, use some himself, and sell the rest to clients at a markup. "It's

just good business," he said. "I'm already there. Why not?"

Why not? Let me count the ways.

First: the legal risk. In most countries, the difference between "personal use" and "intent to distribute" is the difference between a fine and a prison sentence. The moment you start selling, even casually, even "just to friends," you've upgraded your legal exposure from "misdemeanor" to "felony." For what? An extra hundred euros?

Second: the dynamic shift. When you're the plug, the client isn't just a client anymore. Now they have expectations. They want discounts. They want "just a little more." They want to "hang out" while they pick up. They text you at 3 AM on a Tuesday asking if you're "around." Congratulations — you've converted a transactional relationship into a service business with unlimited customer support hours and no overtime pay.

Felix learned this the hard way when a client he'd been supplying started showing up at his apartment unannounced, at all hours, asking for "just a little" and getting aggressive when Felix said no. The client had stopped seeing Felix as an escort and started seeing him as a dealer — and dealers, in the client's mind, don't get to say no to customers.

Third: the math doesn't work. You might make a few hundred extra per month selling. But if you get caught, you lose *everything* — your ability to work,

your freedom, potentially your right to stay in the country you're working in. The expected value of dealing, when you factor in the catastrophic worst-case scenario, is negative. Deeply negative.

If you happen to supply the client — and sometimes it makes sense, because you know your supply is clean and the client's might not be — make sure this is reflected in your fee. Buying drugs from an escort should be like buying a bottle of water at the airport: *convenient, but expensive*. You're taking a risk. You're providing a service that exists outside the normal boundaries of sex work. Charge accordingly.

Better yet: let clients source their own. Your supply is *yours* — for your own consumption, your own control, your own pacing. You're not running a pharmacy. You're running a business.

And never, ever carry more than personal use. The risk-to-reward ratio on dealing is catastrophic. Felix eventually got his life together, but it took a very expensive lawyer and a very uncomfortable year during which he couldn't work, couldn't travel, and couldn't do much of anything except wait for the legal system to decide his future.

I think about Felix whenever a client offers me "extra" to bring something. That extra hundred euros? It's not extra. It's a down payment on a legal nightmare that could cost you everything. You're not a pharmacy. You're not a concierge service for controlled substances. You're a professional

providing a specific, high-risk service. Keep your legal exposure as narrow as your professional offering.

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RULE 13: The Money Conversation Happens Before You're High

This is so obvious it shouldn't need to be a rule. And yet.

I have lost count of the number of escorts I've met — smart escorts, experienced escorts, escorts who should know better — who have ended up working for free, or for dramatically less than their rate, because they "forgot" to discuss money before the drugs kicked in.

The money conversation is awkward. I get it. Nobody wants to interrupt the mood with "so, about my fee." It feels unsexy. It feels transactional. It feels like the opposite of the fantasy you're trying to create.

Guess what: this entire situation is transactional. The fantasy exists *because* of the transaction. The client hasn't forgotten they're paying you — they're just hoping you'll feel too awkward to bring it up. And every minute you delay the conversation is a minute they're winning the negotiation without saying a word.

Here's how I handle it now. The moment I arrive, before any substance enters my body, I say

something like: "Before we get started, let's take care of the practical stuff so we can really relax." I smile. I'm warm. I'm casual. And I'm absolutely not proceeding until the money is in my hand or on the table.

Variations that work:

- "Do you want to handle the fee now so we don't have to think about it later?"
- "I find things go so much smoother when we sort out details first. Then we can just enjoy."
- "I'll take care of the business part now, and then I'm all yours."

Notice the framing: I'm doing this for *us*, for *our* enjoyment, for *your* relaxation. The client isn't being ambushed — they're being freed from the burden of having to pay later. It's a service. I'm providing a service.

If a client pushes back — "can't we do this later?" or "don't you trust me?" — that's not a red flag. That's a red continent. A client who resists paying before the session is a client who was never planning to pay in full. Cut your losses. Leave. The booking that disappears because you asked for payment upfront wasn't a booking — it was a scam.

Cash, upfront, before substances. This is not negotiable.

I want to tell you about a night that could have ended very differently, and the only reason it didn't is that I followed this rule.

A new client in London. Beautiful apartment in Canary Wharf — floor-to-ceiling windows, the Thames glittering below. He was charming. Educated. Had clearly done this before. We talked for twenty minutes before he brought out the substances — long enough that I almost, *almost* let the money conversation slide. I was comfortable. The vibe was good. The drugs looked excellent.

"I'll take care of you at the end," he said, gesturing at the table. "Let's just relax first."

Something in my gut tightened. Not a red flag exactly — more of a yellow one. A hesitation. I had broken this rule before, with regulars I trusted, and it had been fine. But this wasn't a regular. This was a first-time booking in a city where I had no support network within an hour's radius.

"Actually," I said, smiling, "I find things go so much better when we handle the practical stuff first. Then I can really be present with you."

His face did something complicated. Irritation, quickly suppressed. Resignation. And then — and this is the part that still makes my stomach cold — he said: "I don't actually have the full amount right now. I was going to transfer it tomorrow."

Tomorrow. After the booking. After I'd been high in his apartment for four hours. After I'd consumed his substances and was in no position to negotiate. Tomorrow, when he could simply... not transfer it.

I left. I thanked him for his time, I said I hoped we could reschedule when he had the funds available, and I walked out of that beautiful apartment with my sobriety, my dignity, and zero euros — but also zero debt, zero violation, and zero regrets.

Two weeks later, another escort in our circle told me she had been to that same apartment. She hadn't asked for the money upfront. She had done the booking, gotten high, provided four hours of premium chemsex service. When she asked for her fee at the end, the client said he'd "transferred it" and showed her a fake banking app confirmation screen. She was too high to verify it properly. She never saw that money.

Same apartment. Same client. Same scam. The only difference was that I asked the question before the drugs, and she asked it after.

That's not luck. That's a rule. Follow it.

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PART II: THE BODY RULES

Your body is your business. Literally. It's your office, your equipment, your product, your retirement plan, and your vehicle home. Treat it like the million-euro asset it is, because replacing it is not an option.

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RULE 10: Know Your Limits Before You're High

Being high makes it hard to guard your boundaries. I don't mean this metaphorically. I mean the actual neurochemistry of the substances we use makes boundary enforcement physically harder.

MDMA dissolves your sense of separation from others — beautiful in a therapeutic context, dangerous when you need to maintain professional distance. G lowers inhibitions to the point where "yes" and "I guess" feel like the same word. Cocaine makes you feel invincible, which is great for confidence and terrible for risk assessment. Tina makes everything feel intimate and significant, including the client you've known for three hours. None of these substances were designed with sex work in mind. You're operating in an environment where your natural defenses are chemically lowered, and the person across from you may or may not have your best interests at heart.

So draw your boundaries *before*. Before the client arrives. Before the first line. Before the G hits. Write

them down if you need to — there's no shame in having a pre-flight checklist.

What are you absolutely willing to do? What are you absolutely NOT willing to do? What requires an additional fee? What requires a safeword? What makes you leave immediately, no matter how much money is on the table?

I keep a note on my phone. It's not poetic. It says things like:

- "Anal: extra 100, must be discussed before"
- "No choking. Ever. Even if they ask nicely."
- "No filming. No photos. No exceptions."
- "If they touch my neck, I leave."

It's not elegant. It doesn't need to be. It needs to exist, in writing, somewhere I can access even when my cognitive function is at 60%.

Say your boundaries out loud to yourself before the booking. Tell a friend what your limits are for tonight so someone else knows. Communicate them clearly to the client *while you're still coherent*: "Just so we're on the same page — I don't do X, Y is extra, and if at any point I say 'red,' everything stops. Cool?"

Never leave things "open" or "let's just see what happens." That's how you end up doing things you regret for money that doesn't cover the emotional cost. That's how you end up in a taxi at 5 AM feeling like you gave away something you can't get back.

A girl I knew in Buenos Aires — Camila — had a rule: before every chemsex booking, she would call me and read her boundaries out loud. "I don't do anal without a condom. I don't let anyone touch my face. I leave at exactly three hours unless they pay for an extension upfront. If I feel unsafe, I text you the letter X and you call me with a fake emergency."

She did this for two years. Every single booking. I probably heard her boundaries list more times than I've heard my own mother's voice. And in those two years, Camila never once came home feeling violated. Not because clients were all respectful — some weren't — but because she had done the work in advance and knew exactly where her lines were, even when she couldn't see straight.

That's the goal. Not hoping you'll remember in the moment. Knowing, with absolute certainty, because you decided hours ago.

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RULE 11: Serve Your Own Portions

This is the rule that has probably saved more lives than all the others combined.

Especially with G/GBL/GHB.

I need you to understand something about G that nobody tells you when you first encounter it. The difference between a perfect dose and an overdose is measured in *milliliters*. A cap that feels right on Tuesday might knock you unconscious on Thursday

because you haven't eaten. A dose that your friend handles easily might send you to the hospital because your body chemistry is different. The margin is terrifyingly thin, and people die every year because they let someone else pour.

I was at a party in Mykonos once — not working, just social — when a girl I'll call Roxy collapsed. She had let a guy pour her G. "He knows what he's doing," she had said. "He's been doing this for years."

Reader, he did not know what he was doing. He had confused milliliters with something else. Maybe drops. Maybe hope. Whatever it was, Roxy spent the next four hours in a hospital bed, and the guy spent the next four hours pretending it wasn't his fault.

Measure your own intake. Track your own time — minimum one hour between G doses, preferably two. ALWAYS rinse a glass before you use it — there could be residue from the last round that throws off your entire measurement. Use a marked dropper or a syringe. Eyeballing is for people who don't mind occasionally not breathing.

Never, under any circumstances, let a client pour for you. Never accept a drink you didn't watch being made. Never assume "just a cap" means the same thing to them as it does to you. Their cap might be bigger. Their tolerance might be different. Their intentions might not be benign.

I carry a small measuring syringe in my work bag. It cost me less than one euro at a pharmacy. It has saved me more times than I can count. When a client offers me G, I smile, I say thank you, and I measure it myself. If they're offended by this, they're telling me something important about themselves: they value their ego more than my safety. That's not a client I want to be impaired around.

Your measurement. Your timing. Your glass. Your life.

. . .

RULE 12: Know What You're Taking

Don't assume you recognize a substance. Ask what it is. Every time.

I understand the social pressure. You don't want to seem paranoid. You don't want to kill the vibe. You don't want to insult the client's taste or question their judgment.

Let me tell you about a night in Brussels that could have been my last.

A client offered me what he said was MDMA. It looked like MDMA — crystalline, slightly off-white. It didn't smell quite right, but I was tired, I was already a little high, and I didn't want to be difficult. I took it.

Twenty minutes later, I knew something was wrong. The high was not an MDMA high. It was sharper, more dissociative, with edges that didn't feel safe. My heart was doing things hearts aren't supposed to do. I spent the next six hours in a state of controlled panic, pretending to be fine while my body processed whatever I had actually taken.

I never found out what it was. The client swore it was MDMA. Maybe he believed it. Maybe his dealer had cut it with something. Maybe it was a research chemical that looked similar. It doesn't matter. What matters is that I took something without verifying it, and I paid for that decision with six hours of fear.

Now? I ask. Every time. "What is this?" "Where did you get it?" "Have you tried it yourself?" If the client can't or won't answer, I don't take it.

Better yet: get high on your own supply. You know where it came from. You know what it's cut with — or not. You've done it before and know how your body reacts. There is no bravado in accepting mystery substances from strangers. There is only unnecessary risk.

Never try a substance you're unfamiliar with for the first time with a client. First times are unpredictable. First times need a safe environment, people you trust, zero professional pressure, and the freedom to have a bad reaction without worrying about your fee. A paying client's booking provides none of those things.

"I don't know how my body reacts to that, so not tonight" is a complete sentence. Anyone who pushes back is showing you exactly why you shouldn't trust them.

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RULE 15: Personal Care Is Professional Care

When you're high for extended periods, your body keeps score even when your brain doesn't. You might feel fine — better than fine, you might feel transcendent — but your organs are working overtime, your electrolyte balance is shifting, your muscles are tensing, and the bill for all of this will arrive whether you acknowledge it or not.

I learned about personal care from a Brazilian escort named Thiago who worked the European circuit for years. Thiago was famous among our circle for something unusual: at every party, at every booking, in every hotel room, he would quietly and methodically take care of his body. He hydrated on schedule. He took his supplements. He lay down horizontally every eight hours whether he felt tired or not. He stretched. He moisturized. He flossed — yes, *flossed* — even at 4 AM in a stranger's bathroom.

We all made fun of him. "Thiago's spa day," we'd say, watching him unpack his little bag of vitamins and creams. "Thiago's beauty sleep," we'd joke, as

he claimed a corner of the bed for his mandatory rest period.

Thiago is now 42 years old. He still works. He looks a decade younger than his actual age. He has never been hospitalized for a chemsex-related emergency. Most of the people who made fun of him? They're not in the industry anymore — some by choice, some because their bodies gave out.

Here's Thiago's protocol, which I have shamelessly adopted:

- Hydration with electrolytes: Sports drinks, electrolyte packets, coconut water. Plain water isn't enough when you're sweating and not eating. Your cells need sodium, potassium, and magnesium to function. Water alone can actually flush out the minerals you need, making you feel worse.

- Magnesium: For the jaw clenching, the muscle cramps, the generalized tension you don't notice until day three when you can't open your mouth properly. Take it before the booking, not just after.

- Vitamin C and multivitamins: Your immune system is working overtime processing substances. Give it support.

- Food that's easy to consume: Meal replacement shakes, protein bars, fresh fruit, nuts, ice cream, donuts. Whatever you can get down. Calories are calories. Your body is burning through energy at an accelerated rate, and if you don't refuel, it starts consuming muscle tissue.

- Lie down horizontally every 16 hours: Even if you can't sleep. Your feet and legs WILL swell if you don't. The circulatory system was not designed for days of upright activity without rest. Compression socks aren't glamorous but neither is not being able to walk. I have a pair in my work bag. They're black. Nobody notices.

- Shower regularly: Even if you don't feel dirty. Your skin is your largest organ and it's working hard eliminating toxins. Warm water helps circulation. A shower also resets your mental state — it's a punctuation mark in a night that can feel endless.

- Brush your teeth: Meth mouth is real and it is not reversible. Your teeth do not grow back. Your gums do not regenerate. Take the three minutes.

- Moisturizer and eye drops: You look how you feel. Looking fresh affects how clients perceive you, which affects your rates, your bookings, and your confidence.

- Black Cobra or equivalent every 12 hours: If you're working and need to stay hard, circulatory support is not optional. Stimulants constrict blood vessels. Counteract that.

This isn't vanity. This isn't high-maintenance. This is maintenance, full stop. Your body is the vehicle for everything you do — your work, your pleasure, your income, your future. Service it like the luxury vehicle it is, not like a rental car you're planning to total.

PART III: THE MIND RULES

The body can recover from a lot — two days of sleep, some good food, and most physical damage reverses itself. The mind needs different kinds of care, and the drugs that make your body feel invincible can make your mind dangerously soft. The psychological hangover from chemsex work can last weeks longer than the physical one, and nobody talks about it.

RULE 6: Have People You Can Tell the Truth To

You need at least one person — preferably more — who knows what you're doing and won't judge you for it.

Not your clients. Not your regulars who "get it." Not the person you're hooking up with who "doesn't mind" your work. Real people. Friends, chosen family, other workers who understand the game. People you can call at 4 AM and say "I haven't slept in three days, I haven't eaten, I don't feel fully in my right mind, and I think I might have agreed to something I shouldn't have" without shame.

I have a WhatsApp group. It has six people in it. We're spread across four countries and three time zones, which means someone is almost always

awake. The group has one rule: no judgment, no advice unless asked for, and everything shared stays in the group forever. We call it The Kitchen — because it's where we go to warm up.

In The Kitchen, I can say things I can't say anywhere else. "I did a four-hour G booking last night and I still feel like I'm underwater." "The client from Tuesday is texting me and I don't know how to set the boundary." "I think I'm starting to need the drugs to work and I'm scared."

And someone always responds. Not with solutions — solutions can wait — but with presence. "I see you." "I've been there." "You're not alone." "What do you need right now?"

This is not optional. Isolation is the thing that turns a wild weekend into a crisis, a crisis into a pattern, a pattern into an identity. When you have nobody to check in with, it's too easy to let one night become three, one substance become four, one "I'm fine" become the lie you're telling yourself every day.

The antidote to shame is being witnessed. Have witnesses.

If you don't have people like this yet, find them. Other escorts are the obvious starting point — they understand the work, the substances, the specific texture of the problems you're facing. But don't limit yourself. The person you can tell the truth to might be a childhood friend who's never done a drug in their life but loves you unconditionally. They don't

need to understand the details. They just need to be there.

The check-in doesn't need to be dramatic. Before a chemsex booking, I text The Kitchen: "Working tonight, chem. Back by 2AM. Will check in when I'm home." If I don't check in by 2:30, someone texts me. If I don't respond by 3:00, someone calls. If I don't answer, someone takes action. We've never had to escalate, but knowing the system exists makes me feel safer before I've even arrived at the booking.

Build your Kitchen. Fill it with people who want you alive, not just entertained.

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RULE 8: Don't Lay All Your Cards on the Table

Drugs make us chatty. I know this. You know this. Every single person who has ever done a line of cocaine knows this. But knowing it and guarding against it are two different things.

Substances dissolve boundaries that exist for good reason. Suddenly you're telling a client about your childhood, your ex, your real name, where you live, the thing that happened last year that you haven't told anyone, the complicated relationship with your mother, the dream you gave up on, the secret you've been carrying.

Stop. Please. For the love of your future self, stop.

I once listened to an escort — we'll call her Julia — spend forty-five minutes telling a first-time client about the details of her divorce. The custody battle. The financial settlement. The exact neighborhood where her ex-husband now lived. By the end of the booking, this client knew more intimate details about Julia's life than most of her friends did. He was a stranger. He had paid for sex. He now possessed information that could be used to manipulate, extort, or harm her.

Julia realized this the next morning, sober, in a cold panic. She texted me: "Why did I tell him all that?" The answer, of course, was the MDMA. The MDMA had convinced her that this client was her most trusted confidant, her soulmate, the one person who truly understood her. The MDMA was lying. The MDMA always lies about intimacy.

Getting to know you — the real you, the unguarded you, the you that exists behind the professional persona — is a privilege. It's something that *might* happen over time with regular clients who have proven themselves trustworthy over months or years. It is NOT something you gift-wrap and hand to someone on the first date just because the serotonin is flooding your brain.

Keep the conversation light. Keep it positive. Keep it focused on the present, the physical, the shared experience of right now. If a client asks personal questions, you're allowed to deflect, to generalize, to

create a version of your history that's true enough to feel authentic but vague enough to keep you safe.

"My childhood? Pretty normal, actually. What about you?" (You just avoided telling them about your childhood.)

"Relationships are complicated, right? I'm kind of enjoying being solo right now." (You just avoided telling them about your ex.)

"I move around a lot — keeps life interesting." (You just avoided telling them where you live.)

You are not lying. You are curating. You are choosing what to share based on what serves you, not what the drugs are compelling you to release. Information is power, and you don't want a stranger holding power over you that they haven't earned.

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RULE 9: Clients Are Not Looking for a Sob Story

This sounds harsh. I know it sounds harsh. It's also one of the most important truths of this work.

Clients are paying for an escape. They are paying to feel good, to feel desired, to be in the presence of someone who makes them forget their problems for a few hours. They are not paying to be your therapist. They are not paying to hold space for your processing. They are not paying to witness your pain.

I learned this from a client, actually — one of the good ones, a regular in Amsterdam who had been seeing escorts for twenty years. One night, after a booking, we were talking (sober, post-session, the kind of honest conversation that sometimes happens when the transaction is complete), and he said something I've never forgotten:

"The escorts I go back to are the ones who make me feel like everything is okay. Not the ones who are the most beautiful or the most skilled. The ones who, for those few hours, make the world feel manageable. That's what I'm paying for. Not sex. Relief."

This isn't to say you can't be real. You can be real. You can be warm, present, authentic in the moment. But your authenticity should not include your suffering. Your authenticity should be the version of yourself that is thriving — or at least, the version that feels capable of thriving.

Even when a client shares sad or heavy things about themselves — and they will, because intimacy does that, because clients often use escorts as confessional spaces — your job is not to match their energy. Listen. Acknowledge. "That sounds really hard." "I'm sorry you went through that." "Thank you for sharing that with me." Then gently, skillfully, bring the focus back to the body, the present, the pleasure that's available right now.

You are the reminder that pleasure exists. That warmth exists. That their body can feel good. You

are the respite from their problems, not the companion in processing them.

If you need to process something — and you will, because this work surfaces things — do it with your Kitchen. Do it with a therapist who understands sex work. Do it in your journal. Do not do it with the client who is paying for an experience that explicitly excludes your emotional labor.

* * *

RULE 14: Don't Let the Work Consume Your Identity

Who are you when you're not working?

I don't mean "what do you do for money when you're not doing this?" I mean: if someone asked you to describe yourself without mentioning sex work or substances or clients or money, what would you say?

I asked myself this question about two years into working, and I didn't have an answer. I had become so identified with the work that I had forgotten who I was before it. My entire social circle was other escorts. My entire schedule was organized around bookings. My personality had streamlined itself into a single mode: "on." I was always on. Even at brunch with friends, I was performing a version of myself that was charming, entertaining, slightly

theatrical — the escort-self, bleeding into real life because I had forgotten where the boundary was.

A friend sat me down — this was in Barcelona — and asked me a question that felt like an intervention:

"When was the last time you did something that had nothing to do with work, drugs, or recovering from either?"

I couldn't answer. I couldn't remember.

She took me to a Ceramic class. Ceramic. I was terrible at it. I made an ashtray that looked like a wounded animal. I loved every second of it. Because for two hours, I was not an escort. I was not an expert in pleasure. I was just a person with clay on her hands, failing at something that didn't matter.

That ashtray is still on my desk. It's ugly. It's my favorite thing I own. It reminds me that I exist outside of my work, that my value is not dependent on my performance, that there is a self underneath the escort-self who likes making ugly ceramics.

You need something that isn't work. Something that isn't drugs. Something that isn't the client-facing, performance-mode, high-functioning version of yourself that pays the bills. You need a hobby. You need friends who don't work in the industry. You need a skill you're learning that has nothing to do with sex. You need a space where you can be boring, awkward, imperfect, and not performing.

Because here's the truth nobody tells you: the work will end. It might end because you choose to leave.

It might end because your body can't sustain it. It might end because the market shifts or the laws change or you simply age out of the demographic that books you. And when it ends, if there's nothing underneath — no identity, no skills, no relationships, no sense of self beyond "escort" — the crash will be devastating.

Build the thing underneath. Start now. Even if it's just an ugly ashtray. Especially if it's an ugly ashtray.

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PART IV: THE SAFETY RULES

The rules that keep you alive when things go wrong — because eventually, something will go wrong.

This is not pessimism. This is probability. If you do this work long enough, you will encounter a situation that requires preparation, presence of mind, and an exit strategy. The question is whether you'll have one.

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RULE 7: Secure Your Belongings

When you're high, you're not as alert as usual. Things get misplaced. Things get stolen. Things "disappear" while you're in the bathroom. Your phone, your money, your keys, your ID — the things that constitute your lifeline out of any situation — become magnetically attracted to the spaces between couch cushions and the pockets of people who didn't arrive with them.

I have a story about this that still makes me angry, years later.

A client in Barcelona. Lovely apartment, stunning view, excellent drugs — and a habit of "tidying up" while I was in the bathroom. I'd come back and my bag would be "safely put away." My phone would be "charging for you." My money — the money he had just paid me — would be "in a safe place so it doesn't get lost."

Reader, the money was in his pocket. I didn't discover this until I was getting dressed to leave, at which point he claimed he had "no idea" where my envelope had gone. "Are you sure you brought it?"

he asked, with the concerned face of someone who is absolutely lying.

I never got that money back. I had no proof. I was high, tired, and just wanted to leave. He had counted on all of those things.

Now? When someone comes to my space: I tuck away valuables, cash, drugs, and identification BEFORE they arrive. Everything goes into a locked drawer or a hidden compartment. Nothing of value remains visible, accessible, or casually pocketable.

When I visit someone else's space: I carry only the essentials. Phone, keys, cash. One bag. Everything stays in that one bag. The bag stays near the exit — not in a bedroom, not in a locked room, not anywhere I can't grab it in five seconds. If the client wants to "put my things away," I say, "I prefer to keep them with me, thanks." That's the whole sentence. No explanation. No apology.

If you can't grab your bag and leave in ten seconds, you're not in control of your exit. And if you're not in control of your exit, you're not safe.

A variation on this rule: never let a client hold your phone, your keys, or your ID "for safekeeping." That's not safekeeping — that's hostage-taking. Your ability to leave should never depend on someone else's cooperation.

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RULE 16: No One Locks the Bathroom Door

This is a universal chemsex community rule for a reason, and it applies just as much in professional settings as it does at parties.

People get stuck in bathrooms. People pass out on toilets. People slip in showers and can't get up. People take too much G and lose consciousness at the exact moment they were "just going to splash water on their face." People need help and can't call for it, and a locked bathroom door is the difference between "someone checks on you in three minutes" and "someone finds you in three hours."

The bathroom door stays unlocked. Always. No exceptions.

If a client wants privacy for a moment — and that's reasonable, we all want privacy sometimes — they can say so. "I'll be a minute, just need to freshen up." That's fine. Respect it. But the lock stays open, because respect doesn't prevent emergencies, and a polite request for privacy doesn't override the safety of everyone present.

I was at a chemsex party in Berlin where a boy went into the bathroom, locked the door, and didn't come out for forty-five minutes. People knocked. People called his name. Nothing. Eventually someone shouldered the door — which took time, which did damage, which created chaos — and found him unconscious on the floor. He had redosed his G in the bathroom, miscalculated, and gone down.

He survived. But if the bathroom door hadn't been locked, someone would have noticed within five minutes instead of forty-five. Those forty minutes could have been fatal. The lock was the only thing standing between him and timely help.

If someone needs private time for something that requires a lock, and they're in a state where they can make that judgment, that's a conversation. Otherwise: open door, always, for everyone. This rule has saved lives.

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RULE 17: Never Leave Someone Too High Alone

Community care is survival. This is not a suggestion. This is the single most important ethical principle of chemsex culture, and it applies doubly when money is involved.

If someone is too high — a client, another worker, yourself — you do not leave them alone. You do not send them out onto the street. You do not call them a taxi and hope for the best. You stay, or you ensure someone else is staying who is sober enough to monitor them.

Here's how this works in practice:

If a client takes too much and is incapacitated: you are now providing an unplanned service. You didn't sign up for this. It's not your fault. But you're here now, and leaving someone who can't take care of

themselves is not an option. This is the moment where you shift from "escort" to "caretaker." And caretakers get paid.

Make it clear: "I need to stay and make sure you're okay, and that time is billable." If the client is conscious enough to argue, they're conscious enough to pay. If they're not conscious, you're documenting the situation and claiming the time when they recover. You are not a free nurse. You are a professional who encountered a complication, and complications are billable.

If YOU need taking care of — and this happens, because we're human and we miscalculate — make it clear to the client that this is part of the risks of the activity THEY proposed. Your overdose, your bad reaction, your emotional crash — these are not your fault. They're caused by substances that the client wanted to use. The responsibility and the cost are theirs, not yours. You still get paid.

I've had to do this exactly once. I was in London, I took something that hit wrong, and within twenty minutes I knew I was in trouble. I told the client: "I'm not okay. I need to lie down. I need water. I need you to stay with me until this passes." He was surprised. He was probably annoyed. He did what I asked. And when I recovered, two hours later, I reminded him that the full booking fee was still due. "But we didn't—" he started.

"I had a medical emergency caused by the substances you provided," I said. "The fee covers my time. That time included an emergency."

He paid. He wasn't happy, but he paid. He also never booked me again, which was perfectly fine with me — I don't need clients who balk at compensating me for medical events they helped create.

I want to add something here that doesn't fit neatly into any rule but belongs in this chapter because it's about safety of a different kind: the safety of your heart.

This work extracts an emotional toll that the substances can mask for months or years. You might not feel it while you're high. You might not feel it during the recovery. But it accumulates — every client who treated you as a service rather than a person, every booking where you performed intimacy you didn't feel, every night you gave away pieces of yourself that the money didn't quite cover.

The crash isn't just physical. Two or three days after a chemsex binge, when the substances have fully cleared your system and your dopamine receptors are depleted, you might find yourself crying without knowing why. You might feel a profound loneliness that seems disproportionate to your actual circumstances. You might look in the mirror and not recognize the person looking back.

This is normal. It's not weakness. It's neurochemistry plus the accumulated weight of performing emotional labor while chemically altered. Your brain is recalibrating, and in that vulnerable window, everything you've been suppressing surfaces.

What helps:

- Know it's coming. Plan for it. Don't schedule anything important for the third day after a chemsex booking.
- Have someone to call. Your Kitchen. The person who knows what you do. Someone who will sit with you while you cry about nothing and everything.
- Don't make big decisions during the crash. No breaking up with partners. No quitting jobs. No moving cities. Your brain is not in a decision-making state.
- Feed yourself. Carbs, protein, whatever you can keep down. Your brain needs fuel to rebuild the neurotransmitters it just depleted.
- Wait it out. It passes. It always passes. The feeling that it won't pass is a symptom of the crash, not a prediction of your future.

The substances give you wings for a few hours. The crash is the gravity that follows. You can't avoid it. You can only prepare for it, survive it, and remind yourself that it's temporary — that the version of yourself who exists during the crash is not the truest version, just the most depleted one.

RULE 18: Be Realistic About Recovery Time

Plan your downtime like a long-distance flight attendant plans for jetlag. You have a limited number of hours in a week, and the ones you spend recovering are not optional. They are mandatory maintenance. If you don't schedule them, your body will schedule them for you, and your body's scheduling is far less convenient.

Here's my personal formula: for every hour of chemsex work, I need roughly two hours of recovery. A four-hour booking = eight hours of recovery. That's a full day gone. If I book two clients in a weekend, I've lost Monday and Tuesday. If I book three, I'm not functional until Thursday.

This isn't weakness. This is biology. Your nervous system has been chemically hijacked. Your dopamine receptors are depleted. Your sleep cycle has been obliterated. Your body has been performing physical labor while operating on substances that mask fatigue. The crash is coming. It's not avoidable. It's only delayable — and the longer you delay it, the worse it gets.

Financial planning means factoring recovery time into your rates. When you set your hourly price for chemsex work, you're not just pricing the hour you're there. You're pricing the half-day you'll lose afterwards. If your sober rate is 200, your chemsex

rate should be at minimum 300 — the extra hundred covers your recovery hours.

Schedule recovery like you schedule bookings. Block it out on your calendar. "Monday: RECOVERY. No bookings. No social obligations. No gym. Rest." Protect that time. It's not empty space — it's booked by you, for the essential maintenance of your most important asset.

If you have other activities planned — family, sober work, medical appointments, anything that requires you to be present and functional — do NOT work up until the last moment. The comedown will arrive. The exhaustion will hit. You cannot cheat biology forever, and the people in your sober life do not deserve the depleted, irritable, hollowed-out version of you that shows up after a chemsex binge with no recovery buffer.

I learned this in the most painful way possible, and I'm going to tell you about it even though it makes me look bad, because looking bad is less important than you not making the same mistake.

It was my god mother's 60th birthday. I had planned to be there — had booked the flight, had bought the gift, had rehearsed the speech I was going to give at the dinner. But the weekend before, a client I'd been trying to book for months finally confirmed. Three days. Chemsex. His private villa outside the city. The money was... the money was what I'd make in a normal month. "I'll rest on the plane," I told myself. "I'll sleep when I get there."

I didn't sleep. The booking went long — they always go long. The substances accumulated in my system. By the time I got on that flight, I had been awake for the better part of four days. I looked like a ghost. I felt like a ghost that had been hit by a truck. I arrived at my mother's birthday dinner with pupils that were still slightly too large, a jaw that wouldn't unclench, and the emotional range of a dial tone.

My god mother looked at me across the dinner table and said, very quietly, "You're not really here, are you?"

She wasn't angry. She was sad. And that was worse. She had waited for this — her 60th, her niece, the speech I'd been promising — and what she got was a hollow shell of me, present in body only, counting the minutes until I could go back to the hotel and crash.

I gave the speech. I don't remember what I said. I have a video of it somewhere that I've never been able to watch. My mother died three years later. That was her last big birthday. And I was too fried from a chemsex booking to be present for it.

The money from that booking? I don't remember what I spent it on. Probably more drugs. Probably nothing that mattered. But I remember my mother's face. I remember the sadness in her voice. I remember sitting in my hotel room afterwards, alone, coming down, realizing I had traded one of the few genuinely irreplaceable moments of my life for a booking I can barely recall.

Plan your recovery. Protect your important days.
The money will always be there. Your mother's 60th
birthday won't.

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PART V: EMERGENCY KNOWLEDGE

We are escorts. We are not paramedics. We are not
nurses. We do not administer substances to
unresponsive people — at minimum, we need
consent in the moment for whatever care we
provide.

But we DO need to know what to do when things go
wrong. Because they will go wrong, and waiting for
an ambulance isn't always enough. The first five
minutes of an emergency are often the difference
between a bad night and a funeral.

I learned this section the hard way — by being in
situations I was not prepared for, watching people I
cared about suffer because nobody in the room
knew what to do. This is the knowledge I wish
someone had given me before I needed it.

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Too High on G (GHB/GBL)

This is the most common chemsex emergency, and the one you're most likely to encounter. Someone took too much, or redosed too soon because they "didn't feel it" yet, or forgot they had already taken some, or had a different tolerance than they thought.

The person is: incoherent, vomiting, unconscious, or having difficulty breathing. Their skin may feel cold and clammy. They may be sweating profusely. They may have lost bladder or bowel control. This is a medical emergency.

What you do immediately:

- Turn them on their side (recovery position). Airway first, always. If they vomit and they're on their back, they can aspirate.
- Clear their mouth of anything — gum, food, vomit.
- Monitor their breathing. Count breaths. If less than 8 breaths per minute, or if there are gaps of more than 10 seconds between breaths, call emergency services NOW.
- Stay with them. Do not leave them alone for even one minute.
- Keep them warm — blanket, jacket, body heat. G causes temperature dysregulation; they may feel like they're sweating but actually be hypothermic.

What helps if they're still semi-conscious:

- Dairy products — milk, ice cream, yogurt. Dairy helps break down G in the stomach. Easy to swallow.
- Try to make them vomit if they're coherent enough to understand and consent. Getting the remaining G out of their stomach can prevent further deterioration.
- Sips of water with electrolytes, not plain water.

What NOT to do:

- Do NOT give them more drugs to "wake them up" or "balance them out." No cocaine, no speed, no "uppers." Mixing stimulants with a G overdose can cause cardiac complications.
- Do NOT put them in a cold shower. The temperature shock can trigger cardiac events.
- Do NOT leave them to "sleep it off" alone. People die this way. Every year. "Sleeping it off" is how you find someone dead in the morning.

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When to Call Emergency Services

This is the hardest moment in any chemsex emergency. The client is terrified of police involvement. You're worried about your own legal exposure. Everyone in the room is trying to calculate whether this is "bad enough" to justify the consequences of calling for help.

Let me make this simple: if there is ANY question about someone's ability to breathe, call emergency services. Full stop. No debate. No waiting to see if they "come around." No "let's give it five more minutes." Five more minutes is five more minutes without oxygen to the brain.

Calling an ambulance does not mean you have to mention drugs. "My friend has lost consciousness and is having trouble breathing. I don't know what caused it." The paramedics don't need your confession. They need the symptoms. Let THEM figure out the cause — that's their job.

If you're in a country where calling emergency services means police involvement: know this in advance. Have a plan. Know which hospitals are safer. Know which substances are legal or decriminalized and which aren't. This is research you do BEFORE you need it, not during a crisis.

But never let fear of legal consequences override someone's survival. You can deal with the fallout of a police visit. You cannot deal with someone being dead.

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Sweating, Weak, Disoriented
(MDMA/XTC/Speed/Meth)

This person is probably drinking too much water and has diluted the salt in their body. On stimulants, people obsessively hydrate — the jaw clenching, the

dry mouth, the overheating — but they forget electrolytes. They're sweating out sodium and potassium and replacing it with plain water, which further dilutes their blood salt levels. This is called hyponatremia, and it can be fatal.

Signs: confusion, weakness, nausea, headache, disorientation. The person seems "drunk" despite not having drunk alcohol. They may be shivering despite feeling hot.

What helps:

- Instant soup (easy to consume, extremely salty)
- Isotonic sports drinks
- Electrolyte packets dissolved in water
- In a pinch: half a teaspoon of salt dissolved in a glass of water. It tastes terrible. It works.
- Cheese, salted nuts, pretzels — anything salty and edible

What makes it worse:

- More plain water
- Sweet drinks without electrolytes
- "Walking it off"

This is one of the most easily treatable emergencies *if you recognize it*. The problem is that it looks like a generic bad reaction, and people default to "drink more water," which is exactly the wrong response.

Amphetamines: Tense Jaw, Lockjaw

Your jaw is clenched so tight you can't open it properly. Your teeth hurt. Your tongue is pressing against the roof of your mouth like it's trying to escape. This is a common side effect of amphetamines, and while it's not life-threatening, it's extremely uncomfortable and can damage your teeth.

What helps:

- Magnesium supplements — take them before and during the session
- Ice popsicles, cold drinks, anything cold that soothes the muscle tension
- Sugar in liquid form — juice, sports drinks, not solid food that requires chewing
- Conscious awareness: check in with your jaw every fifteen minutes and deliberately unclench
- A mouth guard if you know you're prone to this — they make thin, discreet ones that are barely visible

What makes it worse:

- Chewing gum (seems intuitive, actually overworks the already-exhausted muscle)
- Eating hard or chewy foods
- More stimulants

The jaw will relax when the dose comes down. In the meantime: soft foods, cold drinks, and deliberate relaxation.

Hypothermia Risk (G Specifically)

People on G often sweat profusely but feel cold to the touch. They're vulnerable to hypothermia — the sweating makes you think they're overheating, but their actual body temperature is dropping because G disrupts the hypothalamus, which regulates temperature.

What to do:

- Keep them warm — blankets, body heat, warm (NOT hot) drinks if they can swallow safely
- Move them away from drafts, open windows, air conditioning
- Cover their head — a disproportionate amount of heat is lost through the head
- Monitor: if shivering becomes violent or stops suddenly, the situation is worsening

What NOT to do:

- Do NOT put them in a hot bath or shower. The rapid temperature change can cause cardiac arrhythmia.
- Do NOT give them alcohol to "warm up." Alcohol further dilates blood vessels and accelerates heat loss.
- Do NOT assume sweating means they're overheated. With G, sweating and hypothermia coexist.

When vomiting: recovery position. Airway first.
Never leave them alone on their back. This cannot
be said enough times.

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PART VI: THE PLEASURE PRINCIPLE

— How to Actually Enjoy This

Here's the chapter that almost never gets written in books about chemsex: the one about pleasure. Genuine, transformative, life-affirming pleasure. Because it exists. We all know it exists. That's why we do this. That's why it's hard to stop. The pleasure is real, and pretending otherwise — writing only about risks and dangers and what could go wrong — is dishonest and incomplete.

The question isn't "how do I avoid enjoying this?"
The question is "how do I enjoy it without it
consuming me?"

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The Sweet Spot

Chemsex has a sweet spot — a window of time, usually between one and three hours in, where everything aligns. The drugs have kicked in fully but haven't yet turned on you. The connection with the client is flowing. The discomfort of the initial come-up has passed. The money is on the table, literally or figuratively, and you're no longer thinking about it. Your body feels like a finely tuned instrument. The music is perfect. The lighting is perfect. You are, in this moment, exactly where you want to be.

I've hit the sweet spot on a rooftop in Barcelona at 2 AM, the city spread out below us, the G and the X and the warm night air combining into something that felt like being held by the universe. The client was kind, generous, and genuinely interesting. For those two hours, my work and my pleasure were the same thing. I wasn't performing. I was simply being, and being was paying better than any booking I'd ever had.

That window is real. It's also temporary. This is the part nobody wants to hear.

The sweet spot cannot be extended past its natural lifespan. You can try. God knows we all try. More

drugs, more music, more "just one more hour." But the drugs don't recreate the first feeling — they just delay the comedown and add hours of depleted, mechanical stimulation. The client who was fascinating at hour two is repetitive at hour five. The music that was transcendent is now just loud. Your body has been running on chemicals for hours and the interest payments are coming due.

The art is recognizing the sweet spot while you're in it — fully present, appreciative, aware that this is the peak — and *not trying to extend it*. Let it be what it is. Let it pass naturally. End the booking while the memory is still golden. Go home. Ride the afterglow in your own space, on your own terms.

The escorts who burn out fastest are the ones who chase the sweet spot past its expiration. They're still in the room at 8 AM, six hours past when they should have left, chasing a feeling that stopped being available at 3 AM. The drugs are still there, the money is still technically ticking, but the *magic* is gone, and they're performing a hollow imitation of what was earlier genuine.

Being able to say "that was perfect, and now it's over" is a skill. Practice it.

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Pleasure vs. Oblivion

There's a fundamental difference between "this feels amazing" and "I can't feel anything." The first is

pleasure — an enhancement of experience, an amplification of sensation, a deepening of connection. The second is escape — a flight from something that's too painful to face sober.

Both are valid reasons to use substances. Both are reasons people have used substances for as long as substances have existed. But they require fundamentally different approaches, and confusing the two is where a lot of lives go off the rails.

Pleasure-seeking can be managed with rules, with structure, with the framework this book provides. You can schedule pleasure. You can budget for it. You can incorporate it into a functional life.

Escape-seeking cannot be managed. It can only be *satisfied temporarily*, and it will always come back louder and hungrier. The person who needs drugs to not feel their life, their past, their loneliness — that person can't be fixed by better scheduling. That person needs help that this book can't provide.

Ask yourself honestly: are you getting high because it enhances the experience? Or are you getting high because you can't face the experience sober?

There's a version of this question that's even more uncomfortable: are you doing *sex work* because you can't face something? Is the whole profession a cover for an addiction that would otherwise look like a problem?

I don't have the answer to this for you. I can only tell you that asking the question, regularly and honestly, is the difference between a phase and a trap.

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The Client Is Not Your Lover

This is the hardest line to hold when drugs are involved, and almost everyone I know has crossed it at least once.

MDMA makes you feel connected. It literally floods your brain with oxytocin — the bonding chemical. G dissolves inhibitions and creates physical intimacy that feels emotional. Tina makes everything feel significant, meaningful, destined. Cocaine makes you talkative and confessional. The cocktail of a chemsex booking is essentially a love potion, and you've voluntarily drunk it in the presence of someone who is paying you.

The client is not your lover. The client is paying for a fantasy. You are providing it.

The warmth you feel is real — the chemicals in your brain don't know the difference between a paying client and a genuine romantic interest. That's not your fault. It's not a weakness. It's neurochemistry. But recognizing it as neurochemistry is what lets

you enjoy the feeling without making decisions based on it.

I've watched escorts fall in love on MDMA more times than I can count. "He's different," they say. "He really sees me." "He's not like other clients." And maybe he is different. Maybe he does see you. But you have known this person for four hours, under the influence of powerful bonding chemicals, in a context where he is literally paying for your company. That is not a foundation for a relationship. That is a foundation for a misunderstanding.

Date clients if you want to. I'm not saying don't. But date them sober. Date them during daylight hours. Date them when no substances are involved. If the connection survives the transition from chemsex to real life, it might be real. If it doesn't, it was the drugs — and the drugs are not a sustainable basis for a partnership.

Enjoy the connection in the moment. Let it be beautiful. Let it be warm. Let it be everything it is. Then let it go. The client goes home. You go home. The booking ends. What you experienced was real — and it was also temporary, chemical, and contextual. Both things can be true.

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PART VII: WHEN IT STOPS BEING FUN — Warning Signs

The line between "I do chemsex sometimes" and "chemsex does me" is not a line at all. It's a gradient. It moves so slowly you don't notice until you're on the wrong side of it, looking back at the person you used to be and wondering when you crossed over.

Nobody wakes up one morning and decides to develop a problem. It happens in increments. One

more booking. One more substance. One more "I'll deal with it next week." And suddenly next week was six months ago and you can't remember the last time you worked sober.

Here's how to know when you're approaching the line. These are the warning signs. Take them seriously.

* * *

Warning Sign #1: You're Taking Drugs to Work, Not Working to Take Drugs

Be honest with yourself. Are you accepting chemsex bookings because they pay better and the client is good? Or are you accepting them because you want to get high, and the booking provides the excuse?

There's a simple test: if a regular client who always books chemsex sessions suddenly wanted a sober session, would you still take it? If the answer is no — or if you'd feel disappointed, resentful, or like the booking "wasn't worth it" — you're no longer in control of your relationship with substances.

Another test: when was the last time you turned down a chemsex booking because you didn't want to be high that night, even though the money was good? If you can't remember, that's data.

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Warning Sign #2: You Can't Remember Your Last Sober Booking

If it's been weeks or months since you worked without substances, your ability to choose has atrophied. You're not "choosing chemsex." Chemsex has become the default, and the default is running your life.

Try to remember your last sober booking. What was the client like? How did it feel? Were you present, or were you counting the minutes until you could get high?

If you genuinely can't remember — or if the memory makes you uncomfortable because sober work feels "boring" or "unbearable" now — the drugs have rewired your reward system. Your brain no longer registers sober interaction as worth doing. That's not a preference. That's a dependency.

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Warning Sign #3: Your Sober Life Is Shrinking

The hobbies you used to have. The friends who don't use. The family events you attended. The exercise routine. The creative projects. The quiet afternoons. The things that made you *you* before the work and the substances took center stage — are they still there?

Or has your world narrowed to clients, substances, recovery, and the logistical management of all three?

Look at your calendar for the last three months. How many activities are on there that have nothing to do with sex work or drugs? How many people have you seen who aren't clients or other escorts? How many times have you done something purely for pleasure that didn't involve a substance?

If the answer is "not many" or "I don't remember," your life is shrinking. And shrinking lives are harder to save.

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Warning Sign #4: You're Spending More Than You're Earning

I don't just mean money — though if you're spending more on substances, Ubers, hotels, and impulse purchases than you're bringing in from bookings, that's a problem.

I mean: the total cost of this lifestyle exceeds the total income. Time. Health. Relationships. Reputation. Sleep. Sanity. Ambition. Future options.

You might be making good money on paper. But if you're losing equal value in other areas of your life, you're not making money. You're converting your future health and happiness into present cash, and that conversion rate is terrible.

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Warning Sign #5: You Lie to the People Who Love You

Not the casual "I'm fine" that everyone says when they don't want to talk. I mean the constructed, premeditated lies. The hidden bank account. The secret phone. The story about where you were last night that you rehearsed.

When you're building infrastructure to hide your life from the people who care about you, the part you're hiding is the problem. Healthy lives don't need infrastructure to stay hidden.

* * *

What to Do If This Is You

If you recognize yourself in any of these warning signs, please hear me: you are not broken, you are not weak, and you are not alone. You are having a predictable reaction to a set of circumstances that would challenge anyone. This is not a moral failing. This is a situation that needs to be addressed.

Step One: Tell someone the truth. The person from Rule 6. Your Kitchen. The one who won't judge. Say the words out loud to another human being: "I think I'm losing control of this." Speaking it makes it real. Real things can be faced. Hidden things grow in the dark.

Step Two: Take a break. Two weeks minimum. No chemsex bookings. No "just a little." No "it's been a hard week." Complete abstinence from the substance(s) that are blurring the line. Your brain needs time to reset. Your body needs time to remember what baseline feels like. You will feel terrible for the first few days — that's normal. It's withdrawal, not proof that you need the substance.

Step Three: Work sober during the break. Prove to yourself that you still can. The money won't be as good that week. That's fine. That's not the point. The point is reminding your brain and body that you are capable without the chemical assistance. One sober booking. Then another. Let your nervous system remember that it can do this.

Step Four: After the break, decide. Not "what can I get away with?" but "what actually serves me?" Maybe you go back to chemsex with tighter boundaries and renewed awareness. Maybe you realize you're done with it entirely. Both outcomes are valid. The only wrong answer is not asking the question.

Step Five: If you can't stop — if the two-week break fails, if you find yourself making exceptions, if the pull is stronger than your will — get professional help. There are therapists who specialize in sex work. There are addiction counselors who won't judge your profession. There are support groups specifically for people in the industry. You are not

the first person to need help, and the people who
can help you have heard worse stories than yours.

* * *

PART VIII: THE BUSINESS OF CHEMSEX — Finding, Vetting, and Keeping the Right Clients

This is the chapter that almost nobody writes because almost nobody wants to admit that chemsex work is, fundamentally, a *business*. It's not a lifestyle. It's not a hobby that pays. It's not "just having fun and getting paid for it." It's a specialized service in a high-risk niche, and like any specialized service, the quality of your clients determines the quality of your life.

A good chemsex client is worth their weight in gold — literally, sometimes, given what they pay. A bad chemsex client can cost you days of recovery, your safety, your reputation, or your freedom. The difference between the two often comes down to how you find them, how you screen them, and how you manage the relationship over time.

This chapter is about the business side of what we do — the part that happens before the first line and after the last euro changes hands.

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Finding the Right Clients

Not all platforms are created equal, and the platform you use shapes the clients you attract. Let me break down the landscape as I've experienced it across Europe and Latin America.

Dedicated escort directories (your country's equivalent of Slixa, Tryst, EuroGirls, etc.) tend to attract clients who understand the transaction. They know they're hiring an escort. They expect professional communication. They're generally less likely to push boundaries because they've done this before and they know the etiquette. If you're going to list chemsex services, these platforms are your safest bet.

Grindr and similar apps are a mixed bag. The volume is enormous — you'll get more messages in an hour on Grindr than in a week on a directory. But the quality control is zero. For every genuine client, there are fifty time-wasters, twenty pic collectors, ten people who will ghost the moment you mention rates, and five who are actively hostile to the idea of paying for sex. The upside: when you do find a good client on Grindr, they tend to become regulars because the platform is so chaotic that finding a reliable escort feels like winning the lottery.

I met one of my best regulars on Grindr in London. He messaged me at 11 PM on a Tuesday with the most boring, professional opener I've ever received: "Hi. I see you offer chemsex-friendly bookings. I'm interested in a 4-hour session this weekend if you're available. Here's my number if you'd like to discuss." No dick pic. No "hey." No "looking?" Just a clear proposal from a person who knew what he wanted. That's the goal.

Word of mouth is the holy grail. When a client refers another client to you, they've essentially pre-screened them. The referring client has vouched for the new client's reliability, generosity, and behavior. Some of my most lucrative bookings have come through referrals from regulars who wanted to "share" me with their friends — and because they valued the relationship, they sent me people they knew would treat me well. Cultivate this. A client who refers others is worth more than the sum of their own bookings.

Never accept chemsex bookings from street-level contacts. This should go without saying, but I'm saying it anyway. The risk profile is completely different. The screening is nonexistent. The substances are more likely to be adulterated. The client has no reputation to protect because there's no platform where you can warn other workers about them. This is not snobbery. This is survival math.

* * *

Screening: The Questions You Must Ask

Before you agree to a chemsex booking, you need answers to at least these questions. Not optional. Not "if the vibe feels right." Every time.

"What substances are you planning to use?" — This tells you what you're walking into. If the client mentions substances you don't know or that have a

reputation for causing aggression, paranoia, or unpredictable behavior, decline. You are not obligated to be comfortable with every substance just because you're comfortable with some.

"Have you combined these before?" — A client who's new to chemsex is a different risk profile than an experienced user. The first-timer might take too much, have a bad reaction, panic, or need more caretaking than you signed up for. That's fine if you're prepared for it and charging accordingly. It's a disaster if you assumed they knew what they were doing.

"Do you have your own supply, or are you expecting me to provide?" — Clarify this upfront. If you're providing, your fee reflects that. If they're providing, you need to know what they have and whether you trust their source.

"How long do you anticipate the booking lasting?" — Chemsex bookings tend to expand. A client who says "a few hours" is not a client who has thought about this. A client who says "four hours, with the option to extend at the same hourly rate if we're both enjoying ourselves" is a professional.

"Where is the booking taking place?" — Hotel? Their apartment? Your incall? Each has different safety implications. Hotels are generally safest — there's security, there are cameras, there's accountability. Their apartment means you're on their territory without backup. Your incall means they know where you live. Weigh these carefully.

"Is anyone else going to be there?" — Surprise guests are a hard no. If the client mentions "a friend might join" or "there might be other people," get clear about who, how many, and whether their presence changes your rate. Group chemsex bookings are a different service entirely and should be priced as such.

"Can you share your location with me before I arrive?" — A client who refuses to share their location before a booking is a client who doesn't want you to know where you're going. Red flag.

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Red Flags That Should Make You Walk Away

Some clients identify themselves as dangerous before you've even met them. You just have to pay attention.

- They push back on screening questions. "Why do you need to know all that?" Because I'm entering an altered state in your presence and I'd like to survive the experience. Anyone who resists basic safety questions is not someone you want to be impaired around.

- They try to negotiate your chemsex rate. "Can't we just see how it goes?" No. My rate is my rate. The chemsex rate is higher than my sober rate for reasons I've explained in this book, and if you don't understand those reasons, you don't understand the service you're asking for.

- They mention substances you've explicitly said you don't use. "I know you said no Tina, but I have some really clean stuff." They're already testing your boundaries and you haven't even arrived yet. This does not get better in person.

- They're evasive about where the booking will take place. "I'll send you the address when you're close." "It's in the city center, you'll see." No. Exact address before arrival. No exceptions.

- They ask you to bring substances you'd need to purchase. Now you're not just providing — you're procuring. The legal risk just multiplied.

- They seem intoxicated during the screening conversation. If they're already high during the booking negotiation, they will be worse when you arrive. A client who can't have a coherent conversation about logistics is not a client who can handle a chemsex booking safely.

- They mention previous escorts having "problems" or "overreacting." This is the biggest red flag of all. When a client tells you that their last escort "freaked out" or "couldn't handle it" or "was unprofessional," what they're actually telling you is that something went wrong in their presence and they're preemptively framing it as the escort's fault. Believe the absent escort.

The Regulars: Cultivating Long-Term Chemsex Clients

A good regular chemsex client is the closest thing to passive income this industry offers. They book consistently. They know your rules. They respect your boundaries without being reminded. They pay on time — often in advance — because they value the relationship and don't want to jeopardize it. They refer their friends. They buy you gifts. They become, in some cases, something approaching genuine friends.

Cultivating regulars is an art. Here's what I've learned:

Be consistent in your quality. This doesn't mean be perfect every time — that's impossible when substances are involved. It means be consistently *professional*. Show up on time. Communicate clearly. Don't cancel last minute unless it's an emergency. The drugs may be unpredictable, but your reliability should not be.

Remember things about them. Not in a "notes on my phone" way — though honestly, notes on your phone about regulars' preferences is not a bad idea — but in a "I see you as a person" way. The client who mentioned their divorce three bookings ago will be genuinely touched if you ask how they're doing now. The client who has a specific music preference will notice that you remembered to put on that playlist before they arrived. These small acts of attention

are what convert a one-time booking into a five-year relationship.

Don't get complacent about boundaries. This is the trap that catches experienced escorts. You've seen this client twenty times. He's "different." He's "safe." So you let things slide. You don't discuss the rate upfront anymore because "he always pays." You don't measure your own dose because "he knows my tolerance." And then one night, something goes wrong, and the boundaries you let erode aren't there when you need them. Regulars need the same rules as new clients. The difference is that with regulars, the rules should feel like routine, not negotiation.

Reward loyalty, but don't discount yourself. A regular who books weekly at your full rate for two years is worth more than ten new clients. It's reasonable to offer them something extra — priority scheduling, a slightly longer session for the same rate on special occasions, a small gift. But never, ever reduce your base rate for a regular. That's not rewarding loyalty. That's teaching them that your rate is negotiable. The moment you discount yourself, you've permanently lowered your value in their eyes, and you'll never get the full rate back from them.

Have an exit plan for regulars who become dependent. This happens. A regular who was booking monthly starts booking weekly. Then twice a week. Then they're texting you on your days off,

"just to talk." The chemsex regular becomes emotionally dependent on you in ways that go beyond the professional relationship. When this happens — and you'll know when it does — you have to gently but firmly re-establish the boundary. "I love our sessions, but I need to keep our communication focused on scheduling. Text me when you want to book!" It's warm. It's professional. It protects both of you.

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When to Fire a Client

Yes, you can fire clients. You should fire clients. The ability to walk away from bad money is the difference between a career and a breakdown.

Fire a client when:

- They push a boundary you've clearly stated
- They "forget" to pay or try to renegotiate after the booking has started
- They provide substances that turn out to be different from what they claimed
- They try to film or photograph you without consent
- They become aggressive, verbally or physically
- They make you feel unsafe, even if you can't articulate why — your instincts are data
- They contact you outside of agreed channels, at inappropriate hours, in ways that feel stalking-adjacent

- They talk about you to other escorts in ways that damage your reputation

- They simply stop being worth the money. Some clients drain more energy than their fee justifies. You're allowed to fire them for that.

Firing a client doesn't require a confrontation. It can be as simple as: "I don't think we're a good fit anymore. I wish you the best." Then block them. You don't owe an explanation. You don't owe a second chance. The client who needs to be "fired" has already demonstrated why they need to be fired.

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PART IX: THE ONLINE RULES — Saving Your Time Before the Booking

Half the work in sex work happens before anyone takes their clothes off. The hours spent online — replying to messages, screening, negotiating, being bombarded with questions from people who will never, ever book — can consume your life if you let them.

I have spent entire afternoons on WhatsApp, answering variations of "what are your rates?" from people who had no intention of paying them. I have sent photos to strangers who vanished the moment I mentioned a deposit. I have engaged in lengthy, flirtatious conversations that turned out to be some guy in another country masturbating to the fantasy of booking me, with no actual ability or intention to follow through.

Your online presence is a tool. It is not a hobby. It is not a social life. It is the front desk of your business, and it should be run with the efficiency of a business, not the indulgence of a chat room.

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Use Saved Phrases

You answer the same questions hundreds of times.

"What are your rates?" "Where are you based?"

"What do you offer?" "Can I see more pictures?"

"Are you available tonight?" "Do you do outcalls?"

Type the answers once. Save them somewhere easily accessible — a notes app, a keyboard shortcut, a pinned message in your own chat. Paste them when needed. Personalize the greeting if you want: "Hi! Thanks for reaching out. Here's my info:" and then the saved block.

The client doesn't know or care that it's a saved phrase. They just want the information. You don't get paid per word typed. Every minute you spend composing a custom response to a question you've answered a thousand times is a minute of your life you're donating to someone who may or may not book.

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Pictures Are Not Free Content

There is a specific species of online interaction that every escort knows intimately: the person who only requests pictures. "Can I see more?" "Do you have something sexier?" "Just one more?" "What about from the back?" "Face?" "Face but with your tongue out?"

These are not clients. These are collectors. They have folders on their phones organized by escort. They have no intention of booking, ever. They get off on the interaction itself — the attention, the custom content, the feeling of access. And you are providing it for free.

My response to ongoing picture requests is now automatic: "I share my rates and availability before sending additional photos. Are you looking to book?" Nine out of ten times, they vanish. The tenth time, you might have an actual client. Either way, you saved yourself twenty minutes of unpaid labor.

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Check Their Location First

If someone messages you from another country, another continent, a city you've never been to and have no plans to visit — thank them for the interest and tell them to reach out when you're actually in the same geographic location. There is no booking happening across an ocean. There is no meeting that will take place "when you're next in Berlin" if you have no plans to be in Berlin.

A surprising number of people will message escorts in cities they have no intention of visiting, purely for the thrill of the interaction. Don't feed this. One message: "I'm based in [city]. If you're ever here, let me know!" Done. Move on.

If someone claims to be local but seems suspicious: ask them to share their live location. Real clients who are actually in your city and actually want to meet will do this without hesitation. The ones who won't are either not local, not serious, or not safe.

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Limit What's Available Online

If a client can see everything about you from your ad — every angle, every outfit, every service you offer, every fantasy you cater to — what's left to meet you for?

Your online presence is a preview. A trailer. The full experience is what you're selling, and the full experience includes your presence, your energy, your in-person reality that photos can't capture. Leave something to the imagination. Leave reasons to book.

This isn't about being coy or playing hard to get. It's about understanding that your time and attention are finite resources, and every photo you post, every detail you share, every question you answer online is information you're giving away for free. Save the best for the booking. That's what the booking is for.

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All Conversations Lead to a Concrete Meeting

If a client can't tell you when and where they want to meet, you don't need to tell them what you'll do when you get there.

"A meeting" is the goal of every interaction. Everything else is noise. If they're not ready to schedule, they're not ready to book. Disengage politely and move on.

The conversation should follow a simple arc: greeting → rates/availability → scheduling → confirmation. If it deviates into extended flirtation without a date and time attached, you're being used for free entertainment. You are not a hotline. You are a professional arranging appointments. Treat your chat window like a reception desk, not a dating app.

A phrase I use when someone is dragging out the conversation without committing: "I'd love to continue this in person! When were you thinking of meeting?" It's warm, it's professional, and it forces the issue. They either propose a time or they don't. If they don't, you're done.

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Move to a Real Platform

When communicating via a sex work platform or dating app, move the conversation to a messaging app connected to a real phone number as soon as possible. Real clients use real numbers. Time-wasters use burner accounts, temporary emails, and disappearing profiles.

If someone messages from an unknown number asking for information that's already on your ad: refer them back to your ad. "All my info is in my profile — let me know if you have questions after reading!" This filters out the people who message

every escort in a 50-mile radius without even looking at who they're messaging.

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PART X: THE SOBRIETY ANCHOR — Rule 19

Keep working sober. Keep having sex sober. Do not make your ability to work dependent on substances. This is not about morality. This is not about "clean living." This is about options. This is about leverage. This is about the difference between *choosing* chemsex work because it's profitable and *being trapped* in chemsex work because you can no longer function without it.

If every euro you earn requires you to be high, you are not a sex worker who uses drugs. You are a drug user who does sex work to fund the habit. The distinction matters, not for judgment, but for strategy. A sex worker who uses drugs can stop using drugs and still work. A drug user who does sex work cannot stop either without losing everything. Which one are you?

Have at least one stream of sober income. It doesn't have to be your main income. It doesn't have to be glamorous. It just has to exist — so that when you need to step back from chemsex, you have somewhere to step *to*. Even one sober booking a

month is proof that you can. It's a lifeline to yourself.

I knew a boy in London who worked exclusively chemsex for three years. When he finally decided to stop — his body was giving out, his mind was fraying, he could see the cliff approaching — he discovered he couldn't do a sober booking. Not because he was physically unable, but because he had trained his brain so thoroughly that sex + work = drugs that the equation had reversed: no drugs = no sex = no work. It took him six months and a therapist to undo that wiring. Six months without income. He survived because he had savings. Most people don't.

His name was Alex. We were friends — not close friends, but the kind of industry friends who check in every few weeks, who have keys to each other's apartments for emergencies, who understand each other's lives without needing explanations. Alex was funny, sharp, devastatingly good at his job. He had regulars who adored him. He made more money in a weekend than most people make in a month. And he lost it all — not to drugs, not to a bad client, not to legal trouble — but to the slow erosion of his ability to do anything sober.

I visited him during those six months. He was living off savings in a shared flat in Hackney, trying to remember who he was without the substances and the work. We sat in his kitchen — a grim little room with a flickering fluorescent light and a view of

someone else's laundry — and he told me something I've never forgotten.

"The worst part," he said, stirring a cup of tea that had gone cold an hour ago, "isn't that I can't work. It's that I can't even imagine wanting to have sex. Sober sex. Sex because I want to, not because I'm being paid and not because I'm high. I don't know if I ever knew how to do that, or if I forgot, or if the drugs took it."

He was 28 years old. He was talking about sex — one of the most fundamental human experiences — like it was a foreign language he'd never learned. The drugs and the work had so thoroughly fused in his brain that he couldn't separate pleasure from performance, intimacy from transaction, desire from dosage.

Alex got better. It took time — more time than the six months, if I'm being honest. He found a therapist who specialized in both sex work and substance use, which is like finding a unicorn that also does your taxes. He started with one sober booking a month. Then one a week. He rebuilt his relationship with his own body, his own desire, his own ability to be present without chemical assistance.

But he told me, years later, that he still had to consciously choose sobriety before every booking. It never became automatic. The neural pathway that connected sex and work to drugs had been carved so deep that it would always be there, waiting. He just got better at choosing the other path.

That's what I mean when I say: keep working sober. Not because it's morally superior. Not because chemsex is "bad." But because the alternative — letting the drugs become non-negotiable, letting your ability to function become dependent on substances — is a prison. And once you're inside it, getting out is much harder than never going in.

Don't let yourself become him. Do sober work. Do it regularly. Do it even when the chemsex money is better. Your future self, the one who might need an exit, will thank you for keeping the door open.

And: do not let chemsex work inhibit you from participating in sober activities that matter to you. The family dinner. The friend's birthday. The class you're taking. The gym routine. The Sunday morning market. Some things must be more important than money, because money can't buy them back once they're gone. The chemsex economy will always be there. Your grandmother's 80th birthday won't.

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PART XI: THE MIRROR — Rule 20

Be brutally honest with yourself. Nobody else can do this for you. Nobody else knows what you really do when the door closes. Nobody else tracks what you actually consume, what you actually spend, how many days you've actually been high. The only person who can hold you accountable is the one in the mirror.

• • •

Monitor Your Body

Your mobility. Your flexibility. Your stamina. Your skin. Your teeth. Your heart rate. Your sleep quality. Your appetite.

This work takes a toll, and the toll is often invisible until it's severe. You can feel fine for years while damage accumulates silently. The back pain you're ignoring. The arrhythmia you're dismissing as "just anxiety." The dental issue you're putting off because the dentist is expensive and you'd have to explain the damage. The cognitive fog that's become your new normal.

You know your body better than anyone else. You know when something has changed. Don't gaslight yourself into thinking it's fine. Get things checked. Go to the doctor. Tell them enough of the truth that

they can actually help you. "I use substances recreationally" is a sentence that gives a doctor useful information. "Nothing, I'm fine, I just don't sleep well" is a sentence that delays diagnosis.

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Track Your Use

Mark it on a calendar. When were you last high? On what? For how long? With whom?

When you actually write it down, patterns emerge that your brain has been hiding from you. The frequency that you thought was "occasional" turns out to be "every weekend." The substance you thought you'd "cut back on" is showing up twice as often as last year. The "social" use is now solo. The "work" use is now bleeding into Tuesday.

You don't have to show this calendar to anyone. It's for you. But you cannot address what you don't acknowledge, and denial is the most powerful drug of all.

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Implement Abstinence Periods

Pick a substance. Any substance. The one you're most attached to. Stop taking it for two weeks. A month. Six weeks.

Not "I should probably..." Not "I'm thinking about..."
Actually do it. Mark it on the calendar. Tell
someone. Make it real.

What happens during that period? What feelings
come up that the substance was suppressing? What
situations feel unbearable without it? What do you
reach for instead — food, sex, alcohol, scrolling,
another substance? These are the answers you
need. The substance is not the problem; what the
substance is treating IS the problem. Abstinence
periods reveal what you've been medicating.

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Be Very Honest When Work Is an Excuse to Get High

Not "chemsex pays better." Not "the client wanted
to." Not "it was a special occasion." Those are
justifications. They may be true, but they're also
covering something.

When you accept a chemsex booking, ask yourself:
would I still accept this booking if substances
weren't involved? If the answer is no, the booking is
not about work. The booking is about getting high,
and the work is the cover story you're telling
yourself.

The most dangerous lies are the ones we believe.
This is the lie that catches people: the conviction
that they're "just working" when what they're
actually doing is feeding an addiction through a

socially acceptable channel. "I'm not an addict — I'm a professional. I charge for this." You can charge for it and still be dependent on it. The two things coexist all the time.

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Never Become Financially Dependent on One Source of Income

Reinvest the money you make from chemsex into other aspects of your life. Skills. Education. Savings. Property. A business. Sobriety. Freedom. An exit strategy.

The goal of this work is not to do it forever. The body ages. The market shifts. The substances take their cumulative toll. If you're still doing this at 50, it should be because you want to, not because you have no other options.

I know escorts who used chemsex money to buy apartments, start businesses, pay for degrees, fund their transitions, support their families, build entire lives that will outlast their working years. I also know escorts who spent every dollar they made on more drugs, more parties, more "living in the moment," and who now, years later, have nothing to show for thousands of hours of labor.

The difference is not talent. It's not luck. It's not even sobriety, necessarily. It's the conscious decision to treat this work as a means to an end rather than an end in itself. The money you make

during altered states needs to build something that exists in sober reality. That's not boring. That's strategy.

* * *

PART XII: THE SUBSTANCE GUIDE — WHAT YOU'RE TAKING

This is the chapter I wish someone had handed me in 2013. Not a pamphlet from a clinic that's never seen a chemsex booking. Not a scare sheet that pretends we don't exist. A real guide to what's in your system, what it's doing, how long it's staying, and — most importantly — what happens when it meets other substances or medications already in your body.

Every substance in this chapter, I have encountered in a booking. Either I took it, a client took it, or I watched someone take it and spent the next several hours managing the consequences. This information is not theoretical.

A note before we begin: I am a cosmetic doctor, not a ER Doctor, a pharmacist, or a chemist. I am a worker who has spent over a decade collecting information the hard way. Use this as your starting point, not your final authority. When in doubt, drug checking services like Energy Control in Barcelona will test your actual supply and tell you what's in it. Anonymous. Free. Life-saving.

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| GHB / GBL

Street names: G, Liquid Ecstasy, Gina, Fantasy, Scoop

GHB (gamma-hydroxybutyrate) and its prodrug GBL (gamma-butyrolactone) are the chemsex substance — the one most identified with this scene and the one responsible for the majority of chemsex-related deaths. GHB is a clear, odorless liquid that tastes slightly salty. GBL is an industrial solvent that converts to GHB in the body; it tastes like chemical death and burns going down. Both do the same thing. GBL is roughly 1.6 times stronger and hits faster.

What it does: G is a central nervous system depressant. At low doses it produces euphoria, disinhibition, increased libido, and a kind of warm physical looseness that makes touch feel electric. At medium doses, it causes drowsiness, dizziness, and loss of motor control. At high doses — and the window between "perfect" and "overdose" is terrifyingly narrow — it causes unconsciousness, respiratory depression, coma, and death.

How it feels sexually: G removes inhibition without the forced energy of stimulants. It makes everything feel soft and intense at the same time. This is why clients love it and why chemsex bookings built around G can stretch for hours beyond what either party intended. The drug wants you to stay. It wants you to have another dose. It wants you to merge with the sofa and forget that time exists.

Onset (how long until it hits):

- GHB on empty stomach: 10–20 minutes
- GHB on full stomach: 20–45 minutes (and much less predictable)
- GBL: 5–15 minutes regardless of food

Duration: 1.5–3 hours for the main effects. Residual drowsiness can last longer. The urge to redose hits around the 1.5-hour mark. **DO NOT REDOSE BEFORE 2 HOURS.** I don't care how sober you feel. The G lies about sobriety. Redosing too early is how people overdose.

Other observations:

- The difference between your perfect dose and an overdose is measured in milliliters. 1 ml too much can put you under. 2 ml too much can stop your breathing.
- Alcohol and G are the single most dangerous combination in all of chemsex. They are both CNS depressants. Together they multiply respiratory depression beyond what either would do alone. Multiple deaths every year. **NEVER** mix G with alcohol. Not one drink. Not "just a beer." Never.
- G tolerance builds rapidly with regular use but the lethal dose does not increase proportionally. This means heavy users are actually at **HIGHER** risk of overdose because the gap between "enough to feel it" and "enough to die" shrinks.

- G withdrawal is one of the few drug withdrawals that can kill you. If you've been using G daily, do not stop cold turkey. Seek medical detox.

Mixing with other substances:

Substance	Risk	Notes
Alcohol	☠️ NEVER	Fatal respiratory depression. Multiple deaths documented every year.
Ketamine	☠️ NEVER	Two dissociative depressants. Unconsciousness + respiratory arrest.
Methamphetamine	⚠️ HIGH CAUTION	Stims mask G sedation = delayed overdose. You feel sober, take more G, then crash.
MDMA	⚠️ HIGH CAUTION	Similar masking effect. MDMA energy hides G sedation. Wait minimum 2 hours between.
Cocaine	⚠️ HIGH CAUTION	Same masking issue as other stimulants but shorter duration.
Viagra/Cialis	⚠️ CAUTION	Both lower blood pressure. Combined can cause dangerous hypotension. Reduce G dose.
Poppers	⚠️ CAUTION	Combined vasodilation can crash blood pressure.

With medications:

Medication type	Interaction	What to do
Viagra (sildenafil) / Cialis (tadalafil)	Both vasodilate. Combined = severe hypotension possible.	Reduce G dose by 25%. Do not take Viagra within 2 hours of G.
Estrogen / HRT	No direct dangerous interaction, but G nausea may affect oral hormone absorption.	Take hormones at least 2 hours before or after G.
Testosterone	No known dangerous interaction.	Standard risk. Monitor blood pressure.
HIV ARVs (all classes)	Generally safe. Some ARVs increase sedation from CNS depressants.	Start with lower G dose if on protease inhibitors or efavirenz.

PrEP (Truvada / Descovy)	No known interaction.	Safe to combine.
Spironolactone (T-blocker)	Both affect potassium and can lower blood pressure.	⚠ Stay hydrated. Monitor for dizziness.
Antidepressants (SSRIs, SNRIs)	Possible increased drowsiness.	Lower initial G dose.
Benzodiazepines (Xanax, Valium)	☠ NEVER	Like alcohol – fatal respiratory depression. Absolutely never combine.

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| Methamphetamine (Crystal Meth, Tina)

Street names: Tina, Crystal, Ice, Shabu, Crank

Tina is the queen of chemsex marathons. She's the drug that turns two-hour bookings into twenty-hour odysseys. She's the reason "party and play" exists as a category. And she is, without question, one of the most psychologically addictive substances on earth.

Methamphetamine is a potent central nervous system stimulant. It floods the brain with dopamine — far more than cocaine, far more than MDMA, far more than sex, food, or any natural reward. That flood is what makes her so dangerous: your brain was not designed to experience that level of reward, and once it does, normal life can feel grey by comparison for weeks or months afterward.

What it does: Intense euphoria, hyperfocus, increased energy, massively increased libido, decreased appetite, decreased need for sleep, and a feeling of invincibility that is almost entirely

delusional. You will feel like a genius. You will feel like the most attractive person in the room. You will feel like you could do this forever. None of these feelings are accurate.

How it feels sexually: Tina transforms sex into something obsessive. The physical sensation is heightened — some describe it as every touch being magnified — but the real effect is psychological. Sex on meth becomes compulsive. Hours pass without you noticing. The concept of "enough" disappears. This is why clients who use Tina book long: they genuinely cannot finish, and they genuinely do not want to.

Onset:

- Smoking: 5–10 seconds (immediate rush)
- Injecting: 5–15 seconds
- Snorting: 3–5 minutes
- Oral (bombing): 20–30 minutes
- Booty bumping (rectal): 10–15 minutes, very high bioavailability — dose carefully

Duration:

- Main high: 4–8 hours
- Residual stimulation: 8–16 hours
- Total time before sleep is possible: 12–24 hours after last dose, sometimes longer

This is the most important thing to understand about Tina: you are not sleeping tonight. You are probably not sleeping tomorrow night either. Plan

accordingly. Do not book anything the next day. Do not promise a client you'll stay "just a few more hours" — Tina has already decided your schedule, and she has decided you're staying.

Other observations:

- Meth causes severe vasoconstriction. Your blood vessels tighten. This means higher blood pressure, higher heart rate, and reduced circulation to extremities. Combined with the physical intensity of chemsex, this is a cardiac risk.

- Dehydration and overheating are real dangers. You will not feel thirsty or hot because Tina suppresses those signals. Drink water on a schedule, not by feel. Electrolytes, not just plain water — remember the hyponatremia protocol from Part V.

- The comedown is brutal. Depression, anxiety, exhaustion, and the overwhelming urge to take more to make the bad feelings stop. This is the addiction trap. Plan your comedown before you start: benzos if you have a prescription, sleep aids, comfort food, a safe space, someone who knows what you're coming down from.

- Meth mouth is from dry mouth + grinding + poor oral hygiene during multi-day sessions. Chew gum. Drink water. Brush your teeth even if it feels pointless at 4 AM.

Mixing with other substances:

Substance	Risk	Notes
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| GHB/GBL | ⚠ HIGH | Stims mask G – highest overdose risk. If using both, G doses must be measured and timed, never eyeballed. |

| MDMA | ✗ AVOID | Both are serotonergic stimulants. Serotonin syndrome risk. Extreme cardiovascular strain. Heat stroke danger. |

| Cocaine | ✗ AVOID | Competing stimulants, extreme cardiac load, comedown from both simultaneously is hell. |

| Ketamine | ⚠ CAUTION | K's dissociation can help with the comedown but combined during the high = unpredictable. |

| Mephedrone | ✗ AVOID | Same as MDMA – serotonin syndrome + cardiac risk. |

| Poppers | ⚠ CAUTION | Combined vasodilation/constriction stresses cardiovascular system. |

| Viagra/Cialis | ⚠ CAUTION | Meth raises BP, Viagra lowers it. Combined effect on heart is unpredictable. Start low. |

| Alcohol | ⚠ | Meth masks drunkenness. You'll drink more than you realize. Not the most dangerous, but you'll feel terrible later. |

| Benzodiazepines | ✓ LOW | Common comedown management. Can be life-saving for anxiety/agitation. |

With medications:

| Medication type | Interaction | What to do |

|---|---|---|

| Viagra / Cialis | Meth raises BP/heart rate + Viagra dilates vessels. Combined cardiac stress is significant. | Half your usual Viagra dose. Wait for meth peak to pass first. |

| Estrogen / HRT | Estrogen increases meth's cardiovascular effects slightly. | Standard cautions. Stay hydrated. |

| Testosterone | Possible increased aggression/libido beyond comfortable control. | Be aware of psychological effects. |

Spironolactone	Dehydration risk is higher (spiro is a diuretic).	⚠️ CRITICAL: Drink significantly more water + electrolytes.
HIV ARVs	Ritonavir/cobicistat (boosters) can increase meth levels dangerously.	⚠️ Start with significantly lower meth dose if on boosted regimens.
PrEP	No direct interaction, but meth causes dehydration which stresses kidneys.	Keep PrEP adherence. Drink water.
Antidepressants (MAOIs)	☠️ NEVER	Hypertensive crisis risk with MAOIs + meth. Absolute contraindication.
Antidepressants (SSRIs)	Reduced meth euphoria (SSRIs blunt dopamine response), leading to taking more.	Dangerous cycle. Know this mechanism.
Antipsychotics	Meth may reduce antipsychotic effectiveness. Risk of psychotic episode in predisposed individuals.	⚠️ Serious consideration. Meth can trigger latent psychosis.

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| MDMA (Ecstasy, Molly)

Street names: M, Molly, Mandy, Ecstasy, E, X, XTC

MDMA is the heart-opener. The love drug. It releases serotonin in amounts your brain cannot naturally produce, creating feelings of empathy, connection, and profound well-being. In chemsex bookings, it's often the client's choice when they want the "girlfriend experience" on steroids — intimacy, emotional connection, the illusion that this is something more than a transaction.

What it does: Euphoria, emotional openness, increased sensory perception (music, touch, light),

mild stimulation, jaw clenching, pupil dilation, and a powerful desire for human connection. The "loved-up" MDMA feeling is so potent that escorts often report genuinely enjoying bookings they would normally tolerate — this is the drug talking, and you need to know that.

How it feels sexually: MDMA makes everything feel amazing — and also makes orgasm nearly impossible for most people. It increases sensual pleasure while decreasing genital sensitivity. Touch everywhere on the body becomes erogenous. The experience is more about prolonged sensual connection than goal-oriented sex. This is why MDMA bookings tend to be more emotionally intense and physically less "efficient" than other chemsex sessions.

Onset:

- Oral (crystal/pill): 30–60 minutes
- Snorting (not recommended — burns significantly): 10–20 minutes
- Booty bumping: 15–25 minutes

Pro tip: the come-up can be uncomfortable. Nausea, anxiety, shakiness for about 10–20 minutes. This is normal. It passes. Don't redose during the come-up thinking it's "not working."

Duration:

- Main effects: 3–5 hours
- After-effects (glow or comedown): up to 24 hours

Redosing extends the high but does not bring back the peak. Each redose is diminishing returns with increasing comedown severity.

Other observations:

- MDMA depletes your serotonin. The day after (and sometimes the week after) can include depression, emotional fragility, and exhaustion. This is biological, not personal. You are not weak. Your brain is temporarily low on the chemical that regulates mood. It recovers. But book accordingly — Tuesday after an MDMA weekend is not the day to negotiate anything important.

- Hyperthermia (overheating) is a real danger. MDMA impairs your body's temperature regulation. Dancing + sex + a hot room + MDMA = potential heatstroke. Cool down periodically. Drink water — but not too much (250 ml/hour). Ice packs on the back of the neck work wonders.

- Jaw clenching is universal. Magnesium supplements taken the day before and day of can reduce it.

- MDMA is one of the most adulterated substances on the market. Meth, caffeine, PMA/PMMA (toxic), bath salts — you have no idea what's in that pill unless you test it. PMA/PMMA is especially dangerous: it feels like weak MDMA so people take more, but it's highly toxic and has killed people at doses only slightly above the threshold for effects.

- Frequency matters more than dose for harm.
MDMA should not be used more than once every 4–6 weeks, ideally longer. Every-weekend use causes serotonin depletion that takes months to recover from. The magic disappears — and then so does your baseline mood.

Mixing with other substances:

Substance	Risk	Notes
GHB/GBL	⚠ HIGH	Stimulant hides G. Follow G timing rules exactly. MDMA on the comedown is safer than during the peak.
Methamphetamine	✗ AVOID	Serotonin syndrome risk. Extreme cardiac strain. Heatstroke danger.
Cocaine	✗ AVOID	Cocaine blocks MDMA's serotonin effects (waste of MDMA) + cardiac risk.
Ketamine	✓ LOW	Classic festival combination. K on the comedown can be beautiful. Moderate doses.
Mephedrone	✗ AVOID	Serotonin syndrome. Extreme hyperthermia risk.
Alcohol	⚠	Increases dehydration. Blunts MDMA effects. Feels terrible the next day. One drink max.
Poppers	⚠ CAUTION	Combined vasodilation. Take breaks.
2C-B	✓ LOW	Low-risk combination. 2C-B on the comedown is a common and relatively safe nexus flip.
LSD / Mushrooms	⚠ CAUTION	Candy flip (MDMA + LSD) is intense. Emotional content amplified enormously. Not recommended for work bookings.
Viagra / Cialis	⚠	MDMA itself causes some vasodilation. Combined with PDE5 inhibitors, monitor for low blood pressure.

With medications:

Medication type	Interaction	What to do
Viagra / Cialis	Combined vasodilation.	May cause hypotension (fainting, dizziness). Half dose of Viagra if needed.

Estrogen / HRT	No direct dangerous interaction.	Standard use.
Testosterone	No known dangerous interaction.	Standard use.
Spironolactone	Both can cause mild hypotension.	⚠️ Stay hydrated. Watch for dizziness.
HIV ARVs	Ritonavir/cobicistat can increase MDMA levels dangerously.	⚠️ Lower MDMA dose significantly.
PrEP	No interaction.	Safe.
Antidepressants (SSRIs/SNRIs)	SSRIs block MDMA's effects almost completely by occupying serotonin transporters.	Dangerous: people take more trying to feel something, increasing side effects without euphoria.
Antidepressants (MAOIs)	☠️ NEVER	Fatal serotonin syndrome. Absolute contraindication. Wait minimum 2 weeks after stopping MAOIs.
Lithium	Increased risk of serotonin syndrome.	✗ Avoid MDMA entirely.
5-HTP (supplement)	Taken AFTER MDMA can help serotonin recovery. Taken DURING = serotonin syndrome risk.	Only use 24+ hours after last dose.

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| Cocaine

Street names: Coke, Charlie, Blow, Nieve, Perico, Tussi (not to be confused with 2C-B/tusi/pink cocaine — that's a different substance entirely)

Cocaine has been in the sex work economy since before the term "chemsex" existed. It's less central to the modern chemsex scene than meth or G in many countries, but in Latin America — especially Argentina, Brazil, Colombia — it is still the default party drug that shows up in bookings.

What it does: Euphoria, confidence, talkativeness, increased energy, grandiosity, decreased appetite, local anesthetic effect (numbness). Cocaine is a dopamine reuptake inhibitor — it doesn't flood the brain with dopamine like meth does; it prevents the brain from clearing dopamine that's already there. The result is a shorter, sharper high than meth, with a comedown that arrives fast and demands resolution.

How it feels sexually: Cocaine in sex work is complicated. It makes you feel confident, powerful, and in control. It can increase libido in the short term. But it also causes vasoconstriction — including to the genitals — which means erectile dysfunction for clients and reduced sensation for everyone. In my experience, cocaine is the drug most likely to produce a client who cannot perform but will not stop trying, which is one of the more exhausting scenarios in this work.

Onset:

- Snorting: 1-3 minutes
- Smoking (crack): 5-10 seconds — I include this for completeness; crack cocaine in chemsex bookings is rare and generally a red flag for more chaotic situations
- Oral: 10-30 minutes (much less common route)
- Injecting: immediate — same warning as crack

Duration: 20-40 minutes. Cocaine is the shortest-acting substance in this guide. This is why cocaine

bookings involve constant redosing — and why the financial cost escalates quickly.

Other observations:

- Cocaine + alcohol creates cocaethylene in the liver, a compound more cardiotoxic than cocaine alone and with a longer half-life. This combination is the single most common cause of drug-related cardiac emergencies in emergency rooms. If your client offers coke AND alcohol, be aware that the combination is harder on the heart than either alone.

- The comedown arrives with a crash: irritability, anxiety, paranoia, and a powerful urge to take more. This is what turns a "few lines" into an all-night chase. Set a limit before you start and stick to it.

- Cocaine purity varies enormously. Street cocaine is usually cut with levamisole (a veterinary dewormer that causes skin lesions and suppressed immunity), benzocaine, caffeine, creatine, or worse.

- Frequent cocaine use damages the nasal septum. If you're snorting regularly, rinse your nose with saline between lines. It reduces damage and improves absorption.

Mixing with other substances:

Substance	Risk	Notes
GHB/GBL	 HIGH	Cocaine's short duration makes G timing unpredictable. Caution as with all stimulant + G combos.

Methamphetamine	✗ AVOID	Competing stimulants. Extreme cardiac strain. The comedown from both is punishment.
MDMA	✗ AVOID	Cocaine blocks MDMA effects + cardiac risk. Don't waste your MDMA.
Ketamine	✓ LOW-MOD	"Calvin Klein" combo. K can soften cocaine's sharp edges. Moderate doses.
Alcohol	☠ DANGER	Cocaethylene formation. Cardiotoxic. This kills people. Minimize or avoid.
Poppers	⚠	Combined vasodilation/constriction. Not the worst but not recommended.
Heroin / opioids	✗ AVOID	Speedball. Opposite effects don't cancel – they compound respiratory risk when the stimulant wears off.
Viagra / Cialis	⚠ HIGH	Cocaine constricts + Viagra dilates = confused cardiovascular system. This is a common cause of cardiac events in chemsex.

With medications:

Medication type	Interaction	What to do
Viagra / Cialis	⚠ The most dangerous PDE5 + stimulant combo because cocaine's short cycle means people redose and compound the vasoconstriction.	Avoid Viagra entirely during cocaine sessions if possible. If needed, quarter dose maximum.
Estrogen / HRT	No direct dangerous interaction.	Standard.
Testosterone	Increased aggression/impulsivity possible.	Be aware.
Spironolactone	Dehydration amplified (spiro is diuretic, cocaine increases body temp/heart rate).	⚠ Drink electrolytes.
HIV ARVs	Ritonavir/cobicistat can increase cocaine levels.	Start with lower doses.
PrEP	No interaction.	Safe.
Beta-blockers	☠ NEVER	Beta-blockers + cocaine = unopposed alpha stimulation → coronary vasospasm + heart attack risk. If you take beta-blockers, tell your doctor you use cocaine before getting prescribed. Seriously.

| Antidepressants (MAOIs) | 🦠 NEVER | Hypertensive crisis. |

| Antidepressants (SSRIs) | No dangerous interaction, but cocaine blunts SSRI effectiveness. | – |

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| Ketamine

Street names: K, Special K, Ket, Vitamin K, Kit Kat, Horse Tranquilizer

Ketamine is the wildcard in the chemsex deck. It's a dissociative, which means it separates your mind from your body — and at higher doses, from reality entirely. It has grown enormously in chemsex scenes, particularly in London, Berlin, and increasingly in Latin America.

What it does: At low doses, ketamine produces a floaty, dreamy dissociation, mild euphoria, and body numbness. At medium doses, coordination disappears, speech becomes difficult, and the famous "K-hole" approaches. At high doses, the K-hole: complete dissociation, out-of-body experiences, inability to move or communicate, and experiences that users describe as visiting other dimensions. In chemsex: at low doses it can reduce inhibitions and enhance physical sensation in unusual ways. At higher doses, it incapacitates you — which in a work context is a major safety issue.

How it feels sexually: This is where it gets complicated. Low-dose ketamine can make the body feel less connected to the mind — some workers

report this helps them "float through" bookings they'd rather not be fully present for. But reduced physical sensation also means reduced awareness of pain, boundaries, or injury. You may not notice if something hurts. You may not notice if the client crosses a line. This makes ketamine one of the higher-risk substances for sex work specifically.

Onset:

- Snorting: 5-15 minutes
- Intramuscular injection: 2-5 minutes
- Oral: 15-30 minutes (lower bioavailability, longer duration)

Duration:

- Main effects: 45-90 minutes
- Residual dissociation: 1-2 hours after main effects end

The K-hole, if you reach it, lasts 45-60 minutes of clock time and somewhere between a few seconds and several millennia of subjective time.

Other observations:

- Ketamine is one of the safest substances in terms of direct lethality (overdose alone rarely kills). It IS dangerous in terms of what happens to you while you're unconscious. If you K-hole alone in a room with a stranger, you are completely vulnerable. This is why Rule 12 (emergency contact) and Rule 16 (no locked doors) are especially critical if ketamine is involved.

- Frequent ketamine use causes bladder damage. This is not a scare tactic — it's a well-documented effect called ketamine-induced cystitis. Symptoms: frequent urination, pain, blood in urine. In severe cases, bladder removal. This damage is cumulative and largely irreversible. If you use ketamine regularly, stay hydrated and take breaks between sessions. If you notice urinary symptoms, stop ketamine immediately and see a doctor.
- Ketamine tolerance builds extremely rapidly. What got you high last week may do nothing this week. This tolerance lasts months, not days.
- Ketamine is relatively safe in overdose compared to depressants like G — but mixing ketamine with other depressants makes it lethal through respiratory depression.

Mixing with other substances:

Substance	Risk	Notes
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GHB/GBL	☠️ NEVER	Two dissociative depressants. Respiratory arrest. Deaths on record.
Alcohol	☠️ NEVER	Same as above. Respiratory depression + unconsciousness + aspiration risk (choking on vomit).
Methamphetamine	⚠️ CAUTION	K can smooth meth's aggression. But combined dissociation + stimulation is unpredictable. Low K doses.
MDMA	✓ LOW	Classic. K on MDMA comedown is beautiful. Moderate doses only.
Cocaine	✓ LOW-MOD	Calvin Klein. K softens cocaine's harsh edges. Stagger doses — not simultaneous.
Mephedrone	⚠️	Similar to MDMA combo but less predictable. Caution.

| Poppers | ⚠️ | Combined vasodilation. Brief but intense. Take breaks. |
| Viagra / Cialis | ⚠️ | Ketamine raises blood pressure slightly; Viagra drops it. Confused signals. |

With medications:

Medication type	Interaction	What to do
Viagra / Cialis	Mild to moderate interaction. K's effects on BP are unpredictable.	Standard caution.
Estrogen / HRT	No known interaction.	Safe.
Testosterone	No known interaction.	Safe.
Spironolactone	K is excreted through kidneys. Spiro is a diuretic – stay hydrated.	⚠️ Drink water.
HIV ARVs	Some protease inhibitors can slightly increase ketamine levels. Ritonavir especially.	Start low.
PrEP	No interaction.	Safe.
Antidepressants (SSRIs/SNRIs)	No known dangerous interaction.	Standard use.
Lamotrigine (mood stabilizer)	Can reduce ketamine's dissociative effects.	People may take more to compensate – don't.
Benzodiazepines	Amplify sedation and respiratory depression.	⚠️ Reduce both doses.

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| Mephedrone (4-MMC, Meow Meow)

Street names: M-CAT, Meow Meow, Drone, Bubbles, White Magic

Mephedrone is the substance that brought chemsex to the mainstream in the UK and northern Europe in the late 2000s. It's a synthetic cathinone — chemically related to the khat plant but far more

potent. Think of it as a hybrid: the empathy and warmth of MDMA plus the compulsive redosing of cocaine, with a shorter duration than meth but a similar hook.

What it does: Euphoria, intense empathy, increased energy, heightened sensory perception, increased libido, and a powerful — almost unbearable — urge to redose. Users describe a "rushing" sensation, warmth flooding the body, and a feeling of being completely present and alive. It also causes intense vasoconstriction, which produces cold hands and feet (the "meow meow blues") and, in some people, significant anxiety during the comedown.

How it feels sexually: Mephedrone was practically designed for sex. The combination of MDMA-like empathy, cocaine-like energy, and an almost primal horniness makes sexual experiences on mephedrone feel transcendent. This is why it became the defining substance of the first wave of chemsex. The downside: like cocaine, the high is short and the urge to redose is overwhelming. Sessions can easily turn into compulsive, multi-dose marathons.

Onset:

- Snorting: 5–10 minutes (burns significantly, damages nasal tissue quickly)
- Oral (bombing): 20–45 minutes (smoother come-up, longer duration)
- Booty bumping: 10–15 minutes

Duration:

- Main effects: 1-2 hours (oral), 45-90 minutes (snorted)
- Residual stimulation: 2-4 hours after last dose
- The redose compulsion makes "duration" almost meaningless — people redose continuously for 6-12+ hours

Other observations:

- Mephedrone is extremely moreish — the compulsion to redose borders on involuntary. You will want more before the current dose has even peaked. Set a total amount for the session and put the rest somewhere very inconvenient. No, seriously. Put it in a timed lockbox if you have one.
- The comedown is notoriously brutal. Serotonin and dopamine depletion combine to produce a crash that includes profound depression, anxiety, paranoia, and physical exhaustion. Some users report the worst comedown of any substance.
- Vasoconstriction is severe with mephedrone. Fingers, toes, and nose go cold and blue/purple. This is uncomfortable but usually not dangerous in healthy individuals. However, combined with other vasoconstrictors (like Viagra), it becomes a medical concern.
- Mephedrone is often cut with other cathinones (3-MMC, 4-CMC, 3-CMC) or caffeine. You rarely know what you're getting without testing. 3-MMC has similar effects but is less potent and reportedly less

magical. Some people prefer the gentler 3-MMC for work bookings precisely because it has less compulsion to redose.

- Like MDMA, mephedrone depletes serotonin. Do not use it weekly. Your brain and your baseline happiness both need recovery time.

Mixing with other substances:

Substance	Risk	Notes
GHB/GBL	⚠ HIGH	Same stimulant-masking-G problem as all stimulant combos. Time G doses carefully.
Methamphetamine	✗ AVOID	Extreme cardiovascular strain. Serotonin syndrome possible. Both are potent stimulants.
MDMA	✗ AVOID	Serotonin syndrome danger. Hyperthermia. Do not combine two strong serotonin releasers.
Cocaine	✗ AVOID	Two short-acting, compulsive-redose stimulants. The comedown will be medieval.
Ketamine	⚠ CAUTION	K on the comedown of mephedrone can help. During the peak: unpredictable.
Alcohol	⚠	Increases dehydration, worsens comedown, increases cardiac strain. Minimize.
Poppers	✗ AVOID	Both cause vasodilation. Severe hypotension and fainting. Documented collapses.
Viagra / Cialis	✗ DANGER	Combined vasoconstriction (mephedrone) + vasodilation (Viagra) = cardiovascular confusion. This hospitalizes people.

With medications:

Medication type	Interaction	What to do
Viagra / Cialis	✗ DANGER	See above. Avoid entirely if possible. Mephedrone's severe vasoconstriction makes Viagra interactions more dangerous than with other stimulants.

| Estrogen / HRT | No direct dangerous interaction. | Standard. |

| Testosterone | Possible extreme libido – already high with mephedrone. | Be aware of psychological effects. |

| Spironolactone | Dehydration + vasoconstriction compound circulatory issues. | ⚠️ Drink electrolytes, keep warm. |

| HIV ARVs | Boosted regimens likely increase mephedrone levels. | Start with lower doses. |

| PrEP | No interaction. | Safe. |

| Antidepressants (SSRIs) | SSRIs blunt serotonin effects = people take more = dangerous. | Do not use mephedrone on SSRIs. |

| Antidepressants (MAOIs) | ☠️ NEVER | Fatal serotonin syndrome. Absolute contraindication. |

| Antipsychotics | Risk of triggering psychosis in susceptible individuals. | Caution. |

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| THE MASTER INTERACTION CHART

This is the chart I wanted someone to give me on a single page. Print this. Screenshot it. Memorize it.

Combining Two Substances — Quick Reference:

	GHB	Meth	MDMA	Coke	Ket	Meph
GHB	—	⚠	⚠	⚠	💀	⚠
Meth	⚠	—	✗	✗	⚠	✗
MDMA	⚠	✗	—	✗	✓	✗
Coke	⚠	✗	✗	—	✓	✗
Ket	💀	⚠	✓	✓	—	⚠
Meph	⚠	✗	✗	✗	⚠	—

✓ = Relatively safe at moderate doses

⚠ = Requires caution, dose reduction, timing awareness

✗ = Dangerous. Avoid this combination.

💀 = NEVER. This combination kills people.

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| THE MEDICATION INTERACTIONS MASTER LIST

If you take any of the following medications, read this section before any chemsex booking:

Viagra (Sildenafil) / Cialis (Tadalafil) — PDE5 Inhibitors

The most commonly used medication in chemsex and one of the most dangerous when combined carelessly.

The problem: PDE5 inhibitors dilate blood vessels to increase blood flow. Stimulants (meth, coke, mephedrone, MDMA) constrict blood vessels. Your cardiovascular system gets contradictory commands — dilate AND constrict simultaneously. This causes unpredictable blood pressure, cardiac stress, and in the worst cases, heart attacks or strokes.

Rules for Viagra in chemsex:

- Start with half your usual dose. Quarter if combining with stimulants.
- Wait for the stimulant peak to pass before taking Viagra (2+ hours after dosing).
- Do not take Viagra at the same time as poppers — severe hypotension.
- If you feel chest pain, shortness of breath, or sudden severe headache: stop all activity, call emergency services, and tell them what you've taken.
- The combination of Viagra + cocaine has resulted in cardiac events in men under 40 with no prior heart conditions. This is not a theoretical risk.

Hormone Therapy (Estrogen, Testosterone, Blockers)

Trans and non-binary workers are a significant part of the chemsex economy, and hormone therapy adds specific considerations:

- **Estrogen (oral, injectable, patches, gel):**

Generally safe with most chemsex substances. However, oral estrogen passes through the liver, which also processes most drugs — this can theoretically affect both the estrogen and the drug. Take oral estrogen at least 2 hours before or after dosing if possible. If you vomit during a session (common on come-ups, G overdoses), your oral estrogen may not have been absorbed — take another dose when stable or accept a missed day (one missed dose is safer than double-dosing).

- **Testosterone (injectable, gel):** No dangerous interactions with chemsex substances. However, testosterone + stimulants can amplify aggression, impulsivity, and hypersexuality. Be aware of the psychological combination.

- **Spironolactone (androgen blocker / T-blocker):** CRITICAL. Spironolactone is a potassium-sparing diuretic. It makes you urinate more, which means dehydration risk is higher. Most chemsex substances also cause dehydration. Combined, this can lead to electrolyte imbalances, fainting, and kidney stress.

- Drink electrolyte solutions, not just plain water
- Monitor for dizziness when standing up (orthostatic hypotension)

- Spiro + G can lower blood pressure further — reduce G doses
- **Cyproterone acetate**: Can affect liver function. Liver also processes most drugs. Limit frequency of chemsex sessions if on cypro. Get liver function tested regularly.
- **GnRH agonists (puberty blockers, some HRT regimens)**: No known specific interactions. Standard chemsex cautions apply.
- **Finasteride / Dutasteride (DHT blockers)**: No known interactions. Safe.

HIV Medications

This is one of the most important sections in this book. If you're on HIV treatment, you are not excluded from chemsex — but you need to know how your meds interact.

- **Protease inhibitors boosted with ritonavir or cobicistat** (Darunavir/r, Atazanavir/r, and many others): These boosters inhibit the CYP3A4 liver enzyme — the same enzyme that breaks down MDMA, meth, ketamine, mephedrone, and cocaine. This means drug levels in your blood can spike dangerously. Start with HALF your usual dose of any recreational substance and wait to feel the effects before considering more. Several deaths have been documented from people on boosted ARVs taking their usual MDMA dose.

- **Efavirenz (Sustiva)**: Can increase the psychoactive effects of some substances and has its own CNS effects (vivid dreams, dizziness). Combined with stimulants or psychedelics, the experience may be more intense and more unpredictable.

- **Integrase inhibitors (Dolutegravir, Bictegravir, Raltegravir)**: Generally safe. Few significant interactions with recreational substances. Dolutegravir can increase metformin levels slightly if you're also on metformin for metabolic issues — not directly chemsex-relevant but worth knowing.

- **NRTIs (Tenofovir, Emtricitabine, Lamivudine, Abacavir)**: The backbone of most HIV regimens. Safe with recreational substances. Tenofovir can affect kidney function, which is relevant because dehydration from chemsex stresses kidneys. Stay hydrated.

- **PrEP (Truvada or Descovy)**: One of the few genuinely good news items in this chapter. PrEP has no dangerous interactions with any chemsex substance. However, heavy sessions lasting 24+ hours may cause you to miss a dose. One missed dose does not eliminate protection (especially for daily PrEP), but set a reminder. Dehydration affects kidney function, which affects Truvada processing — drink water.

Practical rule: if you're on boosted ARVs (ritonavir or cobicistat), treat yourself as having zero

tolerance for the first dose of any substance in a session regardless of your usual tolerance. Start low, go slow, stay alive.

Other Common Medications

- **Antidepressants (SSRIs: Sertraline, Fluoxetine, Paroxetine, Citalopram, Escitalopram)**: SSRIs dramatically reduce the euphoric effects of MDMA and mephedrone by blocking serotonin transporters. The danger is NOT a lack of effect — it's that people take more to compensate, increasing the toxic effects (hyperthermia, cardiac strain, serotonin syndrome) without gaining the high. Do not take MDMA or mephedrone if you're on SSRIs. Other substances are less affected.

- **SNRIs (Venlafaxine, Duloxetine)**: Similar to SSRIs but also affect norepinephrine. Same advice: avoid MDMA/mephedrone. Increased risk of serotonin syndrome with serotonin-releasing substances.

- **MAOIs (Phenelzine, Tranylcypromine, Selegiline)**:  NEVER combine with MDMA, meth, mephedrone, or cocaine. Fatal serotonin syndrome or hypertensive crisis. If you're on an MAOI, you should already know this, but I'm saying it again because the consequences are death. Wait a minimum of 2 weeks after stopping MAOIs before using any serotonin-releasing substance.

- **Benzodiazepines (Diazepam, Alprazolam, Clonazepam, Lorazepam):** Often used for comedown management. This is reasonable and can prevent the psychological spiral of stimulant crashes. HOWEVER: benzos + G or benzos + alcohol = 💀 NEVER. Respiratory depression + death. Absolutely never combine benzos with G or alcohol.

- **Antipsychotics (Quetiapine, Olanzapine, Risperidone, Aripiprazole):** These may reduce the effects of stimulants and psychedelics. More importantly, stimulants (especially meth and mephedrone) can trigger psychotic episodes in people with predispositions. If you have a history of psychosis or are on antipsychotics, chemsex stimulants carry significantly higher psychological risk.

- **Mood stabilizers (Lithium, Lamotrigine, Valproate):** Lithium + MDMA or LSD = increased risk of serotonin syndrome and seizures. Avoid. Lamotrigine: no major interactions with most substances but does reduce ketamine's dissociative effects. Valproate: generally safe, but liver processing means caution with heavy sessions.

- **ADHD medications (Methylphenidate/Ritalin, Dexamphetamine/Adderall, Lisdexamfetamine/Vyvanse):** You are already on prescription stimulants. Adding recreational stimulants on top of your daily dose increases cardiac risk, dehydration, and psychosis risk. If you

know a chemsex booking is happening, consider skipping your ADHD medication that day. Do not take recreational stimulants on top of your full ADHD dose.

- **Gabapentin / Pregabalin:** These amplify the sedative effects of G, ketamine, and alcohol. Combined with G or alcohol: respiratory depression risk. ⚠️ Reduce doses.

- **Opioid substitution therapy (Methadone, Buprenorphine):** Methadone and G or alcohol = respiratory depression 💀. Methadone also prolongs QT interval (heart rhythm), and many stimulants also affect heart rhythm. Buprenorphine is safer but still combines with depressants dangerously. If you're on OST, chemsex requires extra medical awareness.

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This chapter is not meant to scare you into abstinence. It's meant to arm you with enough information that you make your choices with your eyes open. The substances are not going anywhere. The bookings are not going anywhere. What can change — what this book exists to change — is whether you walk out of those bookings with your body, your mind, and your future intact.

Know what you're taking. Know what it does. Know what it does with everything else you're taking. And

if you don't know — wait. There will be another booking. There will not be another you.

CONCLUSION: A LIVING TEXT

This torah is not finished. It never will be. New substances will emerge — there are compounds being synthesized right now in labs around the world that will be on the market in five years, and I don't know how they'll interact with this work. New dynamics will develop as the industry shifts, as laws change, as the platforms we use evolve or disappear.

What stays constant is the principle underneath all of this: you matter more than the money, more than the high, more than the client's fantasy, more than anyone else's comfort or convenience.

These rules exist so you can do this work and still have a life. Not one or the other. Both. You deserve the money AND the pleasure AND the safety AND the future. You don't have to choose between making a living and staying alive. These rules are designed to let you have all of it — because you can.

I have buried friends who didn't have rules. I have watched people lose everything — their health, their freedom, their minds, their lives — because nobody told them what I'm telling you now. I have sat in hospital waiting rooms and at memorial services and in the apartments of people I loved who had nothing

left but a habit they couldn't kick and a body that was giving out.

Let me tell you about someone. I'll call her Marina, because her real name belongs to her and the people who loved her.

Marina worked in Barcelona. She was beautiful in a way that made strangers turn around on the street — not the beauty of symmetry or conventional perfection, but the beauty of someone who was fully, unapologetically alive. She laughed like a thunderstorm. She gave the best advice she never followed herself. She could walk into a room and within ten minutes everyone in it would be slightly in love with her.

Marina didn't have rules. She thought rules were for people who weren't having enough fun. "You think too much," she'd tell me, when I tried to talk to her about boundaries and rates and measuring her own doses. "You have to feel it. You have to trust."

She trusted a client who offered her "something new — you'll love it." She didn't ask what it was. She didn't ask where it came from. She trusted him because he was a regular, because he'd always been "nice," because the drugs made trust feel like the only reasonable option.

Whatever she took that night, her body couldn't process it. She had a seizure in his bathroom. By the time the ambulance arrived — he waited too long to call, he was scared, he was high, he was worried

about the police — she had been without oxygen for too long. She survived. But Marina, the Marina who laughed like a thunderstorm, who gave the best advice, who made everyone fall a little bit in love with her — that Marina never came back. What remained was someone who needed full-time care. Who couldn't speak in full sentences. Who didn't recognize the people who loved her.

Marina is why I wrote this book. Marina is why I'm telling you that asking what a substance is, measuring your own dose, and having someone who knows where you are are not optional. They are not "overthinking it." They are not "killing the vibe." They are the difference between a good night and a life that ends, or changes so completely that it might as well have ended.

If Marina had read this book — if someone had told her, clearly and without judgment, what I'm telling you now — she might still be here. She might still be laughing. She might still be giving advice she never followed. She might still be slightly insufferable and completely irreplaceable, the way she was supposed to be for decades longer.

I can't give that back to her. But I can give it to you. So I'm giving it to you. Take it.

This book is my attempt to make those losses mean something. To turn the worst nights I've witnessed into lessons that keep someone else alive. To codify what we all learn, eventually, the hard way — so you don't have to.

Share this book. Steal from it. Add to it. Argue with it. Adapt it to your city, your substances, your specific risks. Translate it into the languages your community speaks. The rules belong to all of us.

If you only remember one thing from everything written here — one single principle to guide you through the chaos — let it be this: the high is temporary. The money gets spent. The client goes home. What remains is you — your body, your mind, your future. Protect those like your life depends on it.

Because it does.

* * *

This text is dedicated to every worker who has ever measured their own dose, counted their own money, enforced their own boundaries, and made their own rules. To the ones we lost — you are not forgotten. To the ones still working — you are not alone. We keep each other safe. We keep each other alive. We are the torah.

* * *

Written by Maxima Caram

*Barcelona - Buenos Aires - Berlin - London -
Amsterdam- Bangkok - Sao Paulo*

2024-2026

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APPENDIX: RESOURCES BY REGION

This is not an exhaustive list. It's a starting point — numbers and organizations that existed at the time of writing. Things change. Verify before you need them.

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Europe-Wide

- European Harm Reduction Network (EuroHRN) — Advocacy and policy work across EU member states. Not a direct service, but their website links to local organizations in most European countries.
- Mainline (Netherlands) — Harm reduction expertise, training, and resources. Particularly strong on stimulant-related harm reduction.
- Techno Plus (France) — Peer-based harm reduction for chemsex communities. French and English resources available.
- Antidote (UK) — LGBTQ+ drug and alcohol support. London-based but offers remote consultations.

- The Dolls Initiative (Netherlands) The Dolls Initiative is a pan-European network of connection, protection, and visibility for trans migrants. We were born from necessity, pain, and the determination to create a space where our community can thrive with dignity and mutual support. I say “we”, because Together with Dinah Bons and Aryelle Freeman Hopelezz we founded and we are directors of “The dolls Initiative” we help trans migrants women with intersectionality’s on sex work, HIV, chemo users and connect directly with health and legal professionals, government institutions. www.thedolls.nl

- ESWA European Sex Work Alliance __ We develop the individual and organizational capacities of sex workers in the region by:

Assessing and consulting members about their needs.

Developing training for individual activists/organisations.

Delivering training and online skills development.

Regional convenings and exchanges.

We represent and amplify the voices of sex workers by:

Listening and being responsive to the needs and perspectives of sex workers, particularly those who are most marginalised, and the evolving issues they face.

Ensuring our staff and leadership are made up of a diverse group of sex workers who come from the movement.

Working directly with sex workers and groups across Europe to build and mobilise an engaged, empowered and effective movement for change by providing training, opportunities for shared learning, developmental assistance, and sub-granting. www.eswalliance.org

- 56 Dean Street (London, UK) — NHS sexual health and chemsex support service. One of the best-known chemsex clinics in the world. Offers counseling, medical support, and harm reduction advice specifically for people combining sex and drugs.

Spain

- Energy Control (Barcelona, Madrid, nationwide) — Drug checking, harm reduction, and information. They'll test your substances and tell you what's actually in them. Anonymous. Free. One of the best harm reduction organizations in Europe.

- Stop Sida (Barcelona) — LGBTQ+ sexual health organization with chemsex-specific resources and support groups.

- Apoyo Positivo (Madrid) — Sexual health and harm reduction, including chemsex support.

- Línea 024 — National suicide prevention hotline. 24/7. Free. Confidential.

Germany

- Sonar (Berlin) — Chemsex-specific harm reduction and counseling. Peer-based. They understand the scene because they're part of it.
- Schwulenberatung Berlin — LGBTQ+ counseling with substance use services.
- Deutsche Aidshilfe — National HIV/STI organization with harm reduction resources.
- Telefonseelsorge — 24/7 crisis hotline: 0800 111 0 111. German-speaking.

United Kingdom

- 56 Dean Street (London) — As mentioned above. The gold standard for chemsex support.
- London Friend — LGBTQ+ drug and alcohol counseling. Chemsex-specific groups available.
- Release — Legal advice for people who use drugs. Know your rights before you need them.
- Samaritans — 24/7 crisis support: 116 123. Free. They won't judge.
- FRANK — Honest information about drugs: 0300 123 6600 or talktofrank.com.

Argentina

- Intercambios Asociación Civil (Buenos Aires) — Harm reduction, drug policy reform, and direct services. They understand the local context.
- Fundación Huésped — Sexual health and HIV services with harm reduction integration.

- Línea 144 — Gender-based violence hotline. 24/7. Free. Nationwide. They can help if a client becomes violent.
 - Línea 135 (Buenos Aires) — Suicide prevention and crisis support. 24/7. Free.
- Brazil
- É de Lei (São Paulo) — Harm reduction organization led by people who use drugs. Peer-based, non-judgmental, grounded in reality.
 - ABGLT — National LGBTQ+ organization with connections to local support services.
 - CVV (Centro de Valorização da Vida) — Crisis and suicide prevention: 188. 24/7. Free. Portuguese-speaking.
 - Disque 100 — Human rights hotline. Can be used to report violence or trafficking.

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General Principles for Finding Help in Any Country

1. Start with HIV/AIDS organizations. They were the first to do community-based harm reduction and they usually have the most up-to-date local information, even if your issue isn't HIV-related.
2. Look for LGBTQ+ health centers. Chemsex is most visible in LGBTQ+ communities, and those health centers are where the expertise has concentrated.

3. Drug checking services are your best friend. If they exist in your city, use them. Knowing what's in your supply is the single most effective harm reduction intervention available.
4. If you're undocumented or working without papers: many harm reduction organizations operate on a strictly anonymous, no-questions-asked basis. They don't report to immigration. They don't share information with police. Their only agenda is keeping you alive.
5. If you need emergency medical care: go to the hospital. Tell them what you took. They are not the police. They need accurate information to treat you. In most countries, medical staff have no obligation to report drug use to law enforcement. Check your local laws, but in a life-threatening situation, your survival comes first.

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What To Say When You Call

If you're calling a crisis line or going to a hospital and you're worried about judgment or disclosure:

- "I had a bad reaction to something I took. I need medical attention."
- "I'm with someone who's unconscious. We've been using substances recreationally."
- "I need to speak with someone who understands chemsex. Do you have a specialist?"

You don't have to disclose that you're a sex worker to get appropriate care. You DO have to disclose what substances are involved, because that determines treatment. "Recreational use" is a phrase that gets you medical information without confessing to anything that might put you at legal risk.

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If you know of resources in your country or city that should be in the next edition of this book, send them to me. This appendix grows with every contribution. We build the map together.



ESWA Health Conference, Brussels, Belgium 2024

Workshop on Chemsex and Sex Work, Maxima V Caram integrated the
Panel of experts on the subject.



The high is temporary. The money gets spent. The client goes home.

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